



HIGHER HEALTH

Higher Education & Training:
Health, Wellness and Development Centre



ANNUAL REPORT 2024/25



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INTRODUCING HIGHER HEALTH

HIGHER HEALTH is dedicated to promoting the health and wellbeing of students at post-school education and training (PSET) institutions nationwide, thereby contributing to their academic success and life beyond graduation.

We are a non-profit organisation and an implementing agency of the Department of Higher Education and Training (DHET), working in partnership with Universities South Africa (USAf), the representative body of public universities, and the South African Public Colleges Organisation (SAPCO), which represents technical and vocational education and training (TVET) colleges. HIGHER HEALTH also enjoys

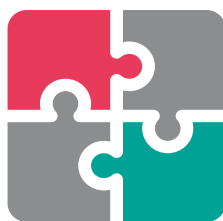
close collaboration with individual universities, TVET colleges and community education and training (CET) colleges, as well as partners in the health, higher education and vocational training sectors.



MISSION

To contribute to academic success among students in PSET through improving and transforming their health, wellness and psychosocial development.





MANDATES

MANDATE 1

Provide the PSET sector with technical guidance on the health, wellness and psychosocial wellbeing of students.

MANDATE 2

Unify the sector response by coordination of stakeholders and role-players.

MANDATE 3

Mobilise resources for the sector and for HIGHER HEALTH's activities and operations.

MANDATE 4

Undertake sectoral research and the monitoring and evaluation (M&E) of programme impact on student health, wellness and psychosocial development.

MANDATE 5

Develop skills and establish capacity building for student health, wellness and psychosocial development in PSET institutions.

MANDATE 6

Conduct teaching and learning while overseeing the implementation of relevant service delivery programmes.

MANDATE 7

Provide easy access to free health and psychosocial services for students at universities, TVET colleges, and CET learning centres.



ABOUT THIS ANNUAL REPORT

HIGHER HEALTH appreciates that our stakeholders and partners may only see the facet of our work closest to them and offers this overview to show how various contributions add value to our total endeavour. Our financial year runs from 1 April to 31 March and our financial statements, together with matters of governance and corporate management, are reported for that period. However, our campus-based programmes are reported mainly for the academic year January to December 2024 because we gear these activities to the campus calendar.

CHAIRPERSON'S MESSAGE



My term as chairperson of the Board ended at the same time as HIGHER HEALTH's financial year and I find myself reflecting not only on 2024/25 but on the entire period that I served.

I joined the Board about a year after the organisation registered as an independent non-profit company and was building its brand under the new name HIGHER HEALTH. Since then, its growth has been exponential and its ability to redefine itself, so that it tackles the very foundations of student health, has been remarkable.

HIGHERHEALTH's current emphasis on the psychosocial wellbeing of students and life skills that support this have been invaluable at a time when South Africa is expanding access to higher education and training. The organisation's accredited, extramural soft skills programmes have the potential to serve as a bridge for students as they transition from school to university or college and once again as they leave campus life for the world of work.

We owe it to our students not only to open the doors of post-school education to them but to provide them with the necessary support to succeed academically and start a career in a tough economic climate. HIGHER HEALTH stands out for its developmental and holistic approach. It makes a major contribution to growing students' resilience, self-esteem, capacity for self-care and community-mindedness.

Within HIGHER HEALTH we appreciate this is pioneering work and a group of universities and colleges in Greater Manchester in the UK confirmed our view when they opted to participate in a pilot run of a customised version of our soft skills programme. Students who completed the pilot spoke enthusiastically about the way learning soft skills had changed their approach to their studies and preparation for the job market. I feel optimistic that HIGHER HEALTH UK will carve out a special place in the crowded British environment.

Funding is a constant challenge for non-profit organisations and HIGHER HEALTH is no exception. However, our leadership has been adept at exploring resources in both the health and higher education and training fields. The Board has helped shape the organisation's approach to funding, ensuring that HIGHER HEALTH's mission and vision remain foremost in the grant partnerships we conclude and that our funders, in turn, receive real value.

While HIGHER HEALTH has gone a long way in improving student access to relevant health services, the ideal of making comprehensive youth-friendly clinical services accessible on all campuses is still unfulfilled. Perhaps digital healthcare consultations will play an increasing role in filling the gaps cost-effectively, as they do in various medical scheme offerings.

During the past year, the Board experienced a degree of renewal as some members stepped down and new ones stepped in. We strengthened the Board representation of TVET colleges because the

sub-sector comprises a large number of institutions and their contribution to post-school education and training is certain to grow in years to come. Prof Deresh Ramjugernath, Vice-Chancellor of Stellenbosch University, has now assumed the role of chairperson. I wish him well in this role and can assure him that an interesting and fulfilling period lies ahead.

Chief Executive Officer Prof Ramneek Ahluwalia and his team deserve full credit for the imagination that has created HIGHER HEALTH's programmes and the dedication required to implement them. My fellow Board members, in turn, have provided wise advice and ably represented an array of stakeholders in the PSET sector. My sincere thanks to all these individuals for their contribution to a successful 2024/25.

Prof Ahluwalia acknowledges HIGHER HEALTH's many partners in his overview of the year. I would simply like to express my appreciation to my colleagues at universities and colleges who have

walked the journey with us, through inevitable highs and lows. Over the years successive Ministers of Higher Education and Training have lent their support to HIGHER HEALTH. We were especially pleased to see Minister Buti Manamela take up this portfolio as he has been a real champion of our programmes throughout his tenure as Deputy Minister.

My association with HIGHER HEALTH was truly a highlight of my time as Vice-Chancellor of Stellenbosch University. I was proud to be part of its mission and am confident it will continue to enhance the health and enrich the learning and life experience of countless students. May you always have the power to surpass your previous best effort!



Prof Wim de Villiers
Chairperson



DEPUTY CHAIRPERSON'S MESSAGE



As principal of a rural TVET college, I formed a relationship with HIGHER HEALTH long before becoming a member of its Board a few months ago. I developed great respect for its work, having witnessed the difference it makes to students' lives through relevant, empowering health and wellness activities.

Many TVET colleges draw their students from the communities immediately around them and are a mirror of local social conditions. A one-size-fits-all approach would not necessarily work for the sub-sector, and we have benefited from HIGHER HEALTH assigning campus care workers to individual colleges to understand our students' needs and tailor interventions accordingly.

TVET colleges were relieved and excited to receive the new intake of HIGHER HEALTH campus care workers in mid-2024 after a period in which funding considerations interrupted such support. Although

college staff members and HIGHER HEALTH employees stepped up to help close the gap, our campus care workers were sorely missed. The new recruits are mostly social work graduates and their training is proving to be a great asset in revitalising health promotion activities.

They have been central to delivery of the Civic and Health Peer Education Programme, which is accredited by the Quality Council for Trades and Occupations and is gradually reaching more TVET colleges around the country. Since TVET courses often focus on technical knowledge and skills, the programme adds real value by building students' confidence in using the soft, people-centred skills that are critical to workplace and entrepreneurial success.

HIGHER HEALTH preaches and lives by the principle of partnership, which has served to bring a range of health and psychosocial services to institutions. As TVET colleges, we could never have developed these services alone, and the results speak to the need to cast our partner search wider, beyond the most obvious sources of support. Colleges have also been inspired to stretch our resources to make joint projects succeed. I would like to acknowledge all those TVET colleges that have prioritised student health and wellness when planning their personnel budgets.

Overall, there were many positive aspects to 2024/25 for HIGHER HEALTH - for example, the expansion of mental health services and the increased utilisation of mobile clinics - and the year closed on an optimistic note. We extend our thanks to everyone across the student health and wellness service chain who helped to make this possible - most especially Prof Ramneek Ahluwalia and his team - and look forward to our continued collaboration with our stakeholders.



Tebogo Kekana
Deputy Chairperson





SECTION 1

CHIEF EXECUTIVE OFFICER'S OVERVIEW

CHIEF EXECUTIVE OFFICER'S OVERVIEW

1



The standout achievements of the 2024/25 financial year were the bedding down of HIGHER HEALTH's accredited extra-curricular Civic and Health Peer Education Programme at universities and colleges across South Africa and a pilot run of the programme in the United Kingdom. These were major landmarks in our journey to help students build critical life skills for the workplace, skills that will also assist them in their studies.

However, a low-key triumph of equal importance was the strong effort to get back to the basics of the HIGHER HEALTH Model. This dramatically improved access to preventive health interventions at our public universities and strengthened clinical and mental healthcare services across the post-school education and training (PSET) sector.

I cannot over-emphasise the importance of **mental healthcare** for students. The reality is that our young people need help: lives are being lost to suicide. Higher Health services such as our helpline and campus clinics may literally save lives.

Our **online soft skills programmes** include two modular options accredited by the Quality Council for Trades and Occupations (QCTO) and leading to NQF Level 5 certification: the six-module Civic and Health Peer Education Programme and the eight-module Civic and Soft Skills Programme.

They are grounded in research that shows career success depends overwhelmingly on the mastery of soft skills, such as empathy and listening, critical thinking, communication, time management, and resilience and self-motivation. (See graphic on page 12 for details of accredited programmes)

We believe that academic success can also be improved by acquiring these skills which should be taught at undergraduate level, not only in post-graduate leadership programmes. Our modular curricula focus on a number of health and social issues relevant to young people, and weave soft skill acquisition into the process of learning about these subjects. Our model is interactive, peer-to-peer driven, and focused on learning by doing.

After two years of curriculum development, online platform refinement and pilot testing, the **Civic and Health Peer Education Programme** launched as a mainstream PSET offering in August 2024 with strong financial backing from the National Skills Fund (NSF). This funding enabled us to employ 75 qualified social workers to deliver the programme. The HIGHER HEALTH Academy, which is responsible for programme development and management, moved into dedicated premises and acquired additional specialised staff. (Read more on pages 12 and 30)

HIGHER HEALTH's relationship with the **United Nations Educational, Scientific and Cultural Organization (UNESCO)** deepened during 2024/25 and our soft skills programme was incorporated into UNESCO's global Transforming MENTalities initiative which tackles gender issues in innovative ways. This endorsement attracted the attention of Universities UK and the Greater Manchester Combined Authority, leading to a trial run of a customised version of our programme in the United Kingdom.

In 2024 several universities and colleges in **Greater Manchester** agreed to partner with HIGHER HEALTH in piloting the programme, known as Skills 4 Living. The first of 950 students signed up in February 2025 and April saw an inspiring launch event with the Greater Manchester Mayor Andrew Burnham. (Read more on page 14)

Meanwhile, back in South Africa, our public universities were proving that our routine, well-established activities could still be exciting. There was a **strong recovery in the number of university students reached** through preventive health and wellness programmes in 2024.

Particularly impressive were the number of condoms distributed in the sub-sector, HIV tests conducted, and students participating in self-assessments of their health risk status. This was due to the universities' recruitment and training of more than 4 000 peer educators to deliver the HIGHER HEALTH Model. It was also enabled by the rebuilding of strong coordination and management structures for our programme at all universities, and by a clearer focus on performance areas funded by our university grants.

Both university and TVET college students had **better access to clinical and mental health services** provided by campus clinics and counselling services, HIGHER HEALTH's mobile clinics, NGO partner organisations, the toll-free helpline, and our team of psychologists and registered counsellors.

Measures that strengthened access to care included:

- Integrating mental health professionals into the service provided by our 11 mobile clinics.
- Gradually training clinic staff to deliver a wider range of health services.

KEY NUMBERS

CAPACITY BUILDING

- Institution management and academics:
2 821
- Peer educators:
5 884

KNOWLEDGE TRANSFER

- Health days organised:
962
- Attendance at CoCurriculum sessions:
225 663
- Completed CSSP and passed assessment:
7 506

STUDENT AGENCY FOR HEALTH

- Total health self-assessments:
327 597

HEALTH PRODUCTS AND SERVICES

- HIV tests and TB and STI screenings:
509 079
- Condoms distributed:
8.65m
- PrEP initiated:
13 293
- ART provided/facilitated:
453
- Contraceptives supplied:
116 456
- Cancer screenings:
89 672

MENTAL HEALTH SERVICES

- Helpline calls and messages:
12 109
- On-campus psychosocial support:
7 461
- Sessions by HH psychosocial team:
31 993

- Strengthening agreements with provincial health departments for the supply of medicines and other health products.
- Moulding our psychologists and counsellors into a cohesive unit managed by a senior psychologist at our head office.
- Introducing a dedicated, multilingual helpline for gender-based violence.

Gratifying though these advances were, there were some concerning **slippages in our performance** at TVET colleges and CET colleges. Across all priority health areas – sexual and reproductive health (SRH), HIV and TB, mental health, GBV, and the health needs of vulnerable student populations – fewer students participated in knowledge-sharing activities and self-assessments of health risks, HIV testing and symptom screening for TB and STIs.

This was mainly due to an interruption in the availability of campus-based HIGHER HEALTH staffers due to the cyclical nature of funding. While this transition between grants was foreseen and plans for new funding were in place, the changeover was not seamless.

The upside to this situation is that the new campus care workers and coordinators, who joined us in July and August, are mainly **qualified social workers** who

have completed four years of training in theory and skills directly relevant to the HIGHER HEALTH Model for promoting health and wellness. These graduates were unemployed due to a shortage of funded positions in the social development sector. After a brief orientation period, the social work recruits proved to be a great asset at campus level.

HIGHER HEALTH owes its **financial stability** and access to resources to expand our work to some key funding partners. More than a decade ago, HIGHER HEALTH took the strategic decision to diversify our funding sources and rely increasingly on domestic institutions. Presently 86% of our income derives from the South African public sector and the remainder from international institutions. Our total grant funding in 2024/25 amounted to R117.5 million, which represents a year-on-year increase of nearly 8%.

Our decision to look inward for funding has proved prescient in the light of the United States' abrupt halt to funding from the President's Emergency Plan for AIDS Relief (PEPFAR). We are sad to see this collaboration ending – because it ran deeper than funding – but the impact on HIGHER HEALTH is limited to an indirect loss. Many HIV, TB and SRH services to which we referred students for care were run by NGOs receiving PEPFAR funding and these services ceased operation early in 2025. We are working with the Department of Health to fill this gap.



SOFT SKILLS DEVELOPMENT AT HOME AND ABROAD

South Africa’s “born free” generation inhabits a massively unequal world and the pressures of poverty undermine the confidence, agency and wellbeing of many students. As a holistic health initiative, HIGHER HEALTH came to appreciate that the empowerment of students through the acquisition of soft skills and promotion of democratic values were critical to student wellness, academic success and workplace readiness.

When we crafted our soft skills course in response to this insight, we never dreamed that it might help students in more affluent countries. But mental stress, civic disengagement and loss of community are widely experienced by young people worldwide. Furthermore, soft skills are increasingly recognised as a key factor in the employability of young graduates. The pilot project we have launched in the UK will confirm whether our soft skills programme can be adapted for different social conditions. (See box on page 14)

OVERVIEW OF HIGHER HEALTH'S ACCREDITED SOFT SKILLS PROGRAMMES

MODULE 1 Civic Education	MODULE 4 Disability	WORKPLACE SOFT SKILLS • Empathy, listening & counselling • Civic responsibility • Conflict resolution • Creativity, critical thinking & problem solving • Decision making • Digital literacy • Communication • Resilience & selfmotivation • Teamwork & leadership skills • Time management & goal setting	QCTO-ACCREDITED PROGRAMMES Civic & Health Peer Education Programme Modules 1 - 6 + Soft Skills • 30 credits • NQF Level 5 • Formal final assessment Civic & Soft Skills Programme Modules 6 - 7 + Modules 7 - 8 + Soft Skills • 60 credits • NQF Level 5 • Formal final assessment
MODULE 2 Gender-Transformative Education	MODULE 5 Comprehensive Sexuality Education		
MODULE 3 Mental Health	MODULE 6 Substance Abuse & Building Resilience		
MODULE 7 Climate Change	MODULE 8 Financial Literacy		

During 2024, the South African pilot run of our first skills programme – the Civic and Health Peer Education Programme – concluded with only nine out of about 1 500 active participants completing all six modules. A much higher number did a few

modules but failed to finish the programme. Focus group discussions suggested a purely online learning format did not suit many participants. Problems of connectivity and digital proficiency were common, and some students struggled at a conceptual level.

HIGHER HEALTH therefore decided to adopt a hybrid learning model for the mainstream implementation of the Civic and Health Peer Education Programme. This entailed campus care workers and coordinators facilitating classroom sessions on each module, and students using the online logbook to complete the practical requirements.

The hybrid system appears to have had the desired effect.

- Students who undertook the course in the second half of 2024 moved swiftly through the programme and 783 out of 2 803 active participants met the final assessment requirements.
- The first cohort of 2025 fared even better. Out of 11 277 active participants, a total of 6 805 passed the final assessment.

With this level of success HIGHER HEALTH is confident it will meet the three-year target agreed with the NSF - namely, that 30 000 young people will achieve the certificate for the full six-module, 30-credit NQF level 5 course over the course of three years. More than that, we believe we have overcome the methodological barrier to offering content that

students find valuable in negotiating entry into adult life.

A critical factor in our turnaround strategy was the engagement of 75 recent social work graduates. Our funding agreement with the NSF stipulates that we will sharpen the skills of these social workers in relation to psychosocial issues impacting on the youth, while simultaneously imparting essential civic and life skills to PSET students. In 2025, our complement of social workers performing as campus care workers and coordinators will increase to 150.

The eight-module, Civic and Soft Skills Programme - which carries 60 credits - will be offered in future as part of two national diplomas. In March 2025, the DHET issued a circular to all TVET colleges confirming that the Civic and Soft Skills course forms part of the experiential learning component of the Nated Report National N Diploma in the fields of engineering and business studies. This is a tremendous incentive for HIGHER HEALTH to make our programme work for students and we are continually fine-tuning our approach to improve outcomes.

HIGHER HEALTH also offers a standalone micro-credential on Digital Literacy and Ethics of AI that students can add to their CVs.



SKILLS 4 LIVING IN MANCHESTER

In Britain HIGHER HEALTH's Skills 4 Living is being assessed for its potential to equip young participants with soft skills that are essential in the workplace, facilitate participation in community life, and enhance young people's wellbeing in a complicated world.

A total of 950 students at 11 universities and colleges in the Greater Manchester area enrolled for the Skills 4 Living pilot project from early 2025. About 45% of these engaged actively in the programme and, at the time of writing this report, 100 certificates had been awarded.

While the pilot has still to be evaluated, initial feedback indicated that more than nine out of 10 participants found the content stimulating and applicable to situations they encountered in real life. Most participants considered that they had made strides in terms of developing empathy, awareness of and respect for diversity, open-mindedness and active listening.

The British programme is managed by a dedicated non-profit company, HIGHER HEALTH UK, the board of which is chaired by paediatric psychiatrist Prof Sandeep Ranote of the University of Salford. Should the pilot confirm that Skills 4 Living delivers on its promise, the programme may be extended to young people not in education, employment or training (NEET) and in correctional facilities.



'I don't think I had a true awareness of how important soft skills are. I can now look at myself and I can champion myself and say: you know what, there's more to me than what I once thought.'

- Chris Rhodes, first year student at University of Salford



'Now that I am applying for graduate jobs, I realise (acquiring soft skills) is so important for the job market and employability - but also for yourself and your confidence. It's really empowering and it's great to add to your CV.'

- Dhruvi Mistry, post-graduate at University of Manchester



PROMOTING HEALTH AND PREVENTING DISEASE

Across its major focus areas – SRH, HIV and TB, mental health and GBV – HIGHER HEALTH emphasises disease prevention and actions students can take to safeguard their health and wellbeing.

HIGHER HEALTH has both informal and structured methods of creating awareness and developing students' understanding of health. The informal activities include campus health days and campaigns,

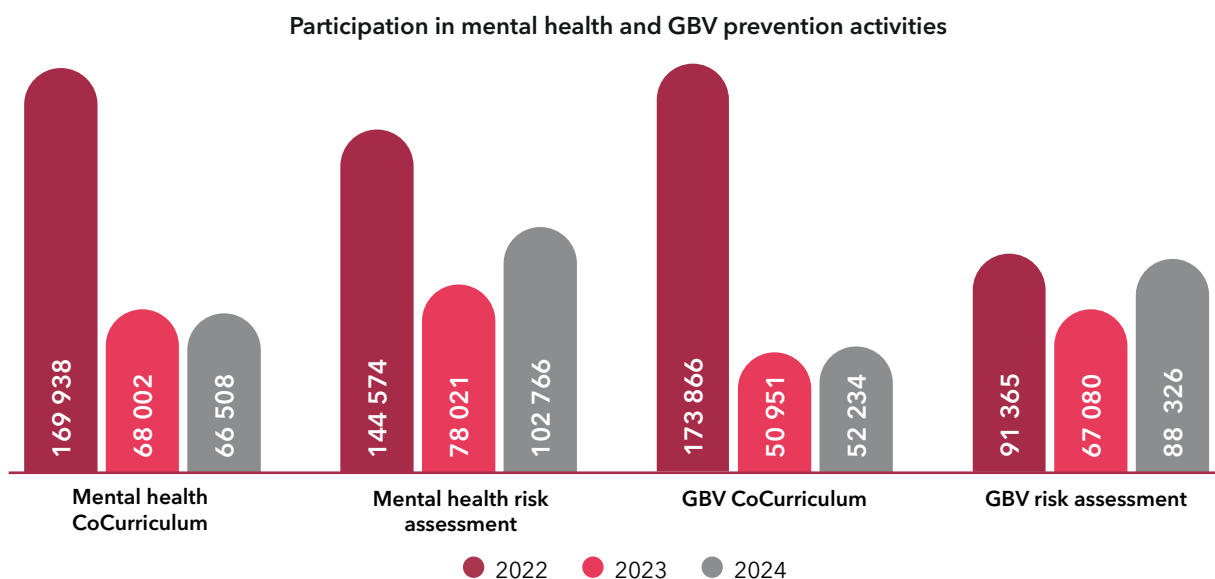
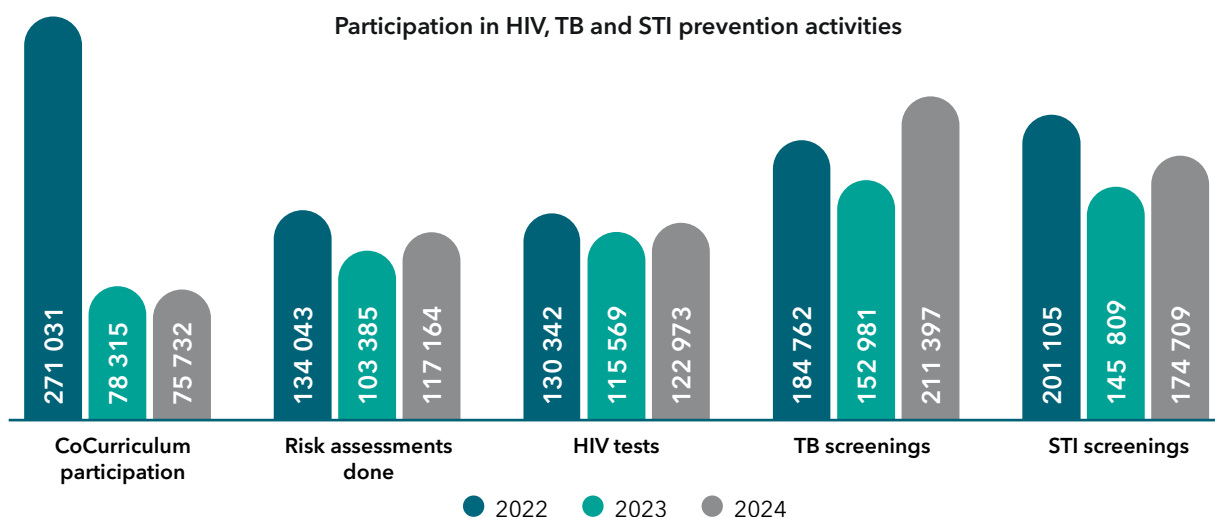
while our more formal classroom learning sessions are known as the CoCurriculum, which is voluntary and comprises modules on various health topics.

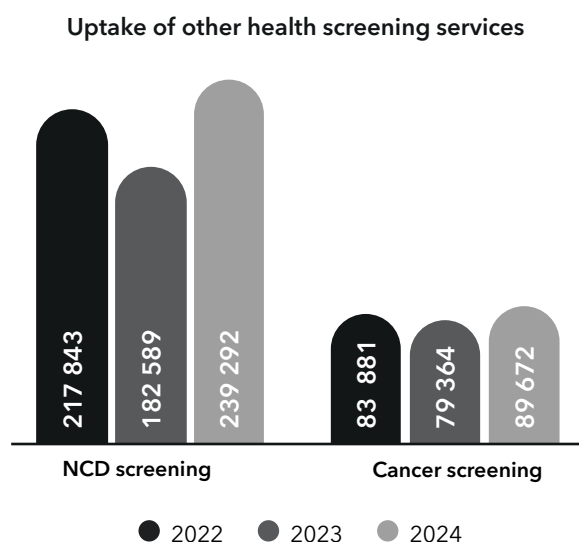
We encourage students to apply their learnings to their own circumstances by completing self-assessments of their health risks and enabling those who self-identify as “at risk” to seek professional help.

We also offer HIV testing and symptom-based screening for TB, STIs and non-communicable diseases because early detection makes treatment easier. These are conducted during campus health days and routinely at campus clinics and mobile clinics managed by HIGHER HEALTH and NGO partners. Where screening suggests the presence of disease, students are referred for further investigation and, where needed, treatment.

As the graphs below show, participation in most of our prevention activities showed a promising recovery in 2024 after a major dip in 2023. However, this rebound for the PSET sector rested almost entirely on the strong performance of the university sub-sector, which offset the underperformance of the TVET and CET sub-sectors. (See Section 3 for figures on sub-sector performance.) For example, the university sub-sector delivered 77% of CoCurriculum sessions on HIV, TB and sexual health, performed 71% of HIV tests and distributed 94% of condoms.

This means TVET and CET college students were under-served in a situation where new HIV infections remain high nationally and are concentrated among young women. It is vital that we get our preventive health interventions in the TVET and CET sub-sectors back on track.





EXPANDING ACCESS TO HEALTH SERVICES

In 2024, the number of students accessing clinic services on campus or through our fleet of 12 mobile clinics was generally higher than the previous year. Mobile clinics performed particularly well in screening for TB, STIs, NCDs and cancer, while campus clinics at universities delivered the bulk of pre-exposure prophylaxis for HIV (PrEP), antiretroviral treatment (ART), and contraceptives. Overall, uptake of PrEP and the demand for contraceptives (excluding condoms) remain lower than might be expected from a young, predominantly female population.

The memorandums of understanding HIGHER HEALTH has concluded with provincial health departments are critical to the improvement of campus and mobile clinic services. Provincial health services provide substantial resources in the form of medication, test kits, contraceptives and other medical supplies. At universities, in particular, this has expanded the range of services offered on campus. We know that one-stop care is preferable to two-step solutions – many a patient is lost to treatment between the making and the fulfilment of a referral.

HIGHER HEALTH's clinical data confirm that the stresses of academic life add to the psychosocial issues that many young people experience. We have therefore prioritised the development of mental health services which are provided to students through different channels. These include the 24-hour toll-free helpline which HIGHER HEALTH established with the help of the South African Depression and Anxiety Group (SADAG) five years ago, various campus counselling

services, and HIGHER HEALTH's team of psychologists and registered counsellors.

Collectively, these services assisted substantially more students in 2024. The major boost came from the HIGHER HEALTH team which more than doubled the number of sessions conducted, from 13 821 in 2023 to 31 993. The inclusion of a senior psychologist in our head office programme management unit has consolidated the work of our psychologists and registered counsellors around the country and helped maximise our resources.

In addition, nearly 600 students received counselling related to GBV and HIGHER HEALTH continued to facilitate workshops under its Transforming MENTalities initiative, focusing on cultural and psychosocial factors among men that perpetuate high levels of GBV.

PARTNERSHIPS FOR PROGRESS

Historically, partnerships have been the sturdiest pillars of HIGHER HEALTH and they remain our strength as we look to the future.

When South Africa emerged from the pre-ART era and the mortal threat of HIV lifted from the PSET sector, we began to explore how we could support student health in a more comprehensive way. We were fortunate to find partners that shared our view that physical and mental health were major factors in the academic success of students and that developmental health programmes enhanced the contribution graduates could make in the workplace and in their communities.

Many organisations mentioned below have been with us on this decades-long journey.

Our relationship with the institutions that comprise the PSET sector – universities, TVET colleges and CET colleges – literally sustains the work of HIGHER HEALTH. This collaboration unfolds at many levels – from management who sanction and guide our activities, to staff who coordinate and provide skilled services, and student volunteers who bring vigour and creativity to health promotion activities.

HIGHER HEALTH's positive working relationship with the Ministry of Higher Education and Training has been a major asset. When leadership at this level demonstrates confidence in our work and backs our

awareness campaigns, we feel the positive ripples across our stakeholder network. Behind the scenes, we benefit from the professional knowledge and technical advice of senior officials in the Department of Higher Education and Training (DHET).

We receive similar “donations” of specialist knowledge and practical insight from countless organisations and individuals in South Africa and the global health and higher education community. In the past two years UNESCO has brought new thinking to HIGHER HEALTH, especially in relation to gender equality. UNESCO has established an official partnership, with consultative status, with HIGHER HEALTH and I feel fortunate to have been elected to serve on the Global UNESCO Liaison Committee for the next two years.

Funding organisations are indispensable partners. HIGHER HEALTH receives most of its funding from South African government departments and other public entities, and significant additional support from international funding organisations. We strive to show our appreciation by delivering the expected outputs, keeping credible data for reporting purposes and maintaining good financial management. The fact that some donors have continued to fund us over successive grant cycles suggests that these partnerships are in good shape.

The public health sector – from the national department, through provincial departments to health districts and local clinics – plays a significant role in supporting our preventive and treatment interventions. We feel they have accepted us as an integral part of the primary healthcare system, and provide us with significant health supplies, help build the capacity of our clinical staff and accept referrals of students for treatment beyond the ability of our own clinics. Non-profit health organisations, funded by PEPFAR to work in the areas of HIV, TB and SRH, have been valuable operational partners and HIGHER HEALTH sorely misses their presence in the wake of the cut-off of PEPFAR funding early in 2025.

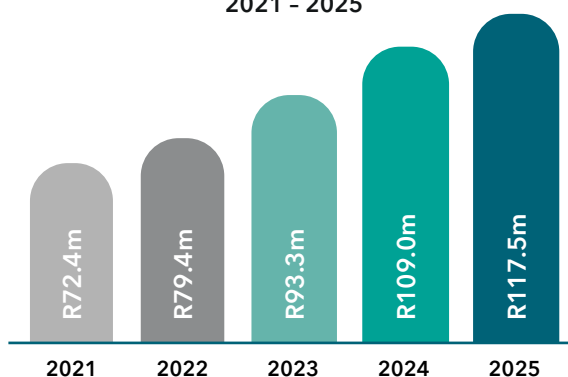
FUNDING AND FINANCIAL STABILITY

HIGHER HEALTH continued to see growth in its annual income in 2024/25 and the strengthening of its domestic funding base. Total income from grants for the year amounted to about R117.5 million, exceeding the previous year's figure of R109.0 million by nearly 8%.

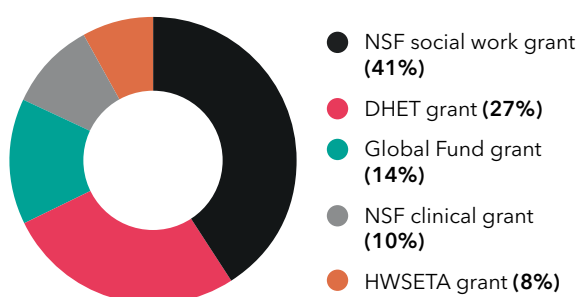
The new three-year National Skills Fund (NSF) grant – for building social workers’ youth development skills and delivery of the Civic and Health Peer Education Programme – contributed R48.2 million to our total income in 2024/25, becoming our largest single source of funding. Most of our funders have supported us over multiple funding cycles. However, we are mindful that funders’ circumstances may change and sustainability requires that we continually diversify our sources of funding. We also strive to identify new synergies with existing funders, so that we produce the proverbial “win-win” results.

During the year HIGHER HEALTH created and filled the position of Executive Manager: Special Projects and Business Development to expand strategic and funding partnerships. This enabled us to initiate discussions with 15 organisations beyond our normal funding base in the early months of 2025, with a view to submitting written proposals in the coming financial year. Our focus has been sector education and training authorities (SETAs) and several private sector corporations.

**Growth in HIGHER HEALTH grant income
2021 - 2025**



Sources of grant funding 2024/25



Note: The German Corporation for International Cooperation (GIZ) also provided financial assistance amounting to less than 1% of the total.

HIGHER HEALTH concluded the year with an operating surplus of R6 223 112. Human resources costs absorbed 51% of operating expenditure, consultant fees 10% and travel expenses 9%.

Summary of income and expenditure

	2025	2024
	R'000	R'000
REVENUE	117 548 142	109 005 130
Other operating income	2 279 839	2 998 086
Other operating gains (losses)	226 581	(160 819)
Operating expenses	(116 611 649)	(109 624 967)
Operating surplus	3 442 913	2 217 430
Interest income	1 038 620	28 020
Finance cost paid	(808 218)	(703 268)
Fair value gains	2 549 797	1 531 508
Surplus for the year	6 223 112	3 073 690

MANAGEMENT DEVELOPMENTS

Human resources

HIGHER HEALTH is a fast-growing organisation with a high volume of contract posts. These characteristics place considerable demands on our human resources (HR) team. At the end of March 2025, our employee headcount was 223, representing year-on-year growth of 31%. In August 2024, in the space of a few weeks, no fewer than 75 social workers were appointed to our establishment. Speed in filling posts after the award of a major grant is critical, not only to meet the service targets stipulated in the grant agreement but also to minimise downtime in HIGHER HEALTH's support to institutions during grant changeovers.

Despite these pressures, the HR team has found time to strengthen systems for employee management. The new organogram has streamlined the organisation, removing the tier of regional managers and adding specialist managers at head office to provide appropriate support to health professionals operating from provincial offices.

The HIGHER HEALTH Academy has been constituted as a functional entity, with the range of technical expertise needed to develop and sustain online learning programmes.

Monitoring and evaluation

Our ability to produce credible, verifiable data on programme performance is crucial to our relationship with funding partners. HIGHER HEALTH has had difficulty finding the right solution for digitising the collection, reporting, national aggregation and analysis of data. In the past year, the digitisation project was carefully scoped and a tender published. The tender was under adjudication as the financial year ended and will be awarded in 2025/26.

In the meanwhile, we devote time and effort to improving the quality of our manual collection system through the training of programme delivery staff, the review and updating of standard operating procedures, and provision of regular feedback on errors identified in the verification process.

Communication and stakeholder relations

Strategic communication plays a vital role in the life of HIGHER HEALTH, not only in terms of profiling our work to the public but also in building cohesion across our complex implementation system. As usual, in 2024/25, we engaged with mass media through public relations outreach and paid partnerships.

But our emphasis in this period was expanding production of our communication materials – including blogs, short-format videos, and live streams – and developing systems to distribute these to our intended audiences. Our communication team had the exciting opportunity to plan and help execute the launch of the Skills 4 Living programme in the UK, applying their South African experience successfully in an unfamiliar setting.

ACKNOWLEDGMENTS

HIGHER HEALTH's contribution to the health and wellbeing of PSET students would not be possible if other role players were not equally passionate about this cause and able to bring to the table resources, experience and insights. I extend my deep appreciation to each one who has helped make this collaborative venture successful.

It is impossible to mention every contribution, but I would like to acknowledge some of our major supporters.

We are deeply appreciative of the enduring interest that the Ministry of Higher Education and Training has taken in the work of HIGHER HEALTH. We also thank the Director-General of DHET and senior managers in the department who do so much to facilitate our programmes, as well as senior managers of the NSF who have supported new directions in HIGHER HEALTH.

We appreciate the support of vice-chancellors of universities and principals of TVET and CET colleges and the practical involvement of diverse members of management and staff at institutions across the country. We are also grateful to Universities South Africa (USaf) and the South African Public Colleges Organisation (SAPCO) which enable our work and contribute to the governance of HIGHER HEALTH.

We would like to recognise the important role of student leadership and peer educators in mobilising student participation in health and wellness activities.

We have been fortunate to work with funders who have not only provided financial resources but have taken time to understand HIGHER HEALTH and the wellness challenges of students, and have been willing to back novel solutions suited to the PSET sector. We have benefited particularly from the NSF's commitment to innovation in skills development.

Campus health services and our mobile clinic fleet receive invaluable assistance from provincial and district health services as well as non-governmental and community-based organisations. We would like to thank them for their collegial cooperation.

In recent years, HIGHER HEALTH has engaged increasingly in international forums with colleagues doing similar work to us in other countries. The experience has been encouraging and enriching. During 2024, HIGHER HEALTH truly became part of the UNESCO NGO family and we are grateful for the welcome we received. In the UK, the generosity and interest of academics, educators, policy makers and students not only made the launch of Skills 4 Living possible, but ensured it was a stimulating journey.

As always, I extend my thanks to members of the HIGHER HEALTH Board who keep a watchful eye on our work and offer guidance and encouragement. Our longstanding chairperson, Prof Wim de Villiers, vacated this position as he retired as Vice-Chancellor of Stellenbosch University at the end of March 2025. We will greatly miss his wisdom and the grace with which he headed the Board.

Finally, to my management team and all staff members of HIGHER HEALTH, every year in our organisation is a challenge because we are a dynamic organisation, unafraid of trying new ways of working. We do not always succeed immediately – but we get up and try again until we find the path. Your resilience and commitment are deeply appreciated.



Dr (Prof) Ramneek Ahluwalia
Chief Executive Officer

Health and Wellness



SECTION 2

STRATEGY, OPERATIONAL
MODEL AND STRUCTURE

1

PROMOTING HEALTH IN PSET SETTINGS

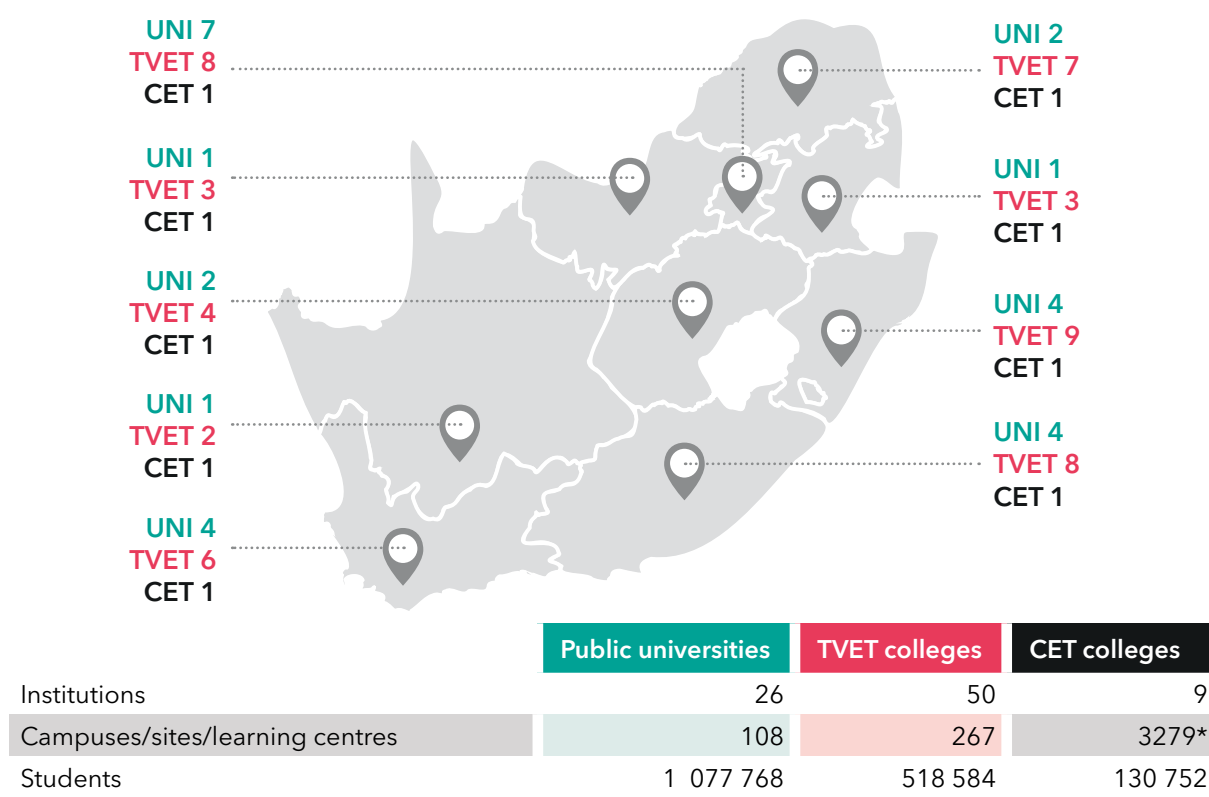
HIGHER HEALTH's strategic planning is informed by a vision that we share with the PSET sector: advancing the development of young people so that they are academically successful, physically and mentally healthy, and emerge from their years of study eager and ready to contribute to society.

While health programming for students is always secondary to the core academic purpose of universities and colleges, its significance cannot be overstated. Our students may be intellectually promising, but many will face significant personal challenges during their years of study, including threats to their health. It is critical that we give them the tools to survive these crises and fulfil their promise. We regard personal empowerment as one of the most valuable tools for academic and career success.

HIGHER HEALTH's emphasis is on the promotion of health, the prevention of illness, and timely intervention to treat both physical and psychosocial conditions. We strongly encourage young people to be the agents of their own wellbeing by increasing their understanding of health, utilising available services, and contributing to a supportive campus environment. We assist universities and colleges to improve students' access to health services that are welcoming and relevant to their needs.

HIGHER HEALTH'S five-year strategy and operational model create a clear framework for enabling health and wellness programmes across the three main categories of PSET institutions – public universities, TVET colleges and CET colleges. Our structure and staffing plan allow us to tailor implementation to fit the different requirements of these sub-sectors.

Distribution of PSET institutions



Notes: *This is according to the DHET website and includes satellite centres. However, Statistics on post-schooled education in SA notes data on the CET sub-sector are not completely reliable. Enrolment at public universities increased by nearly 1% in 2024 while enrolment in the TVET and CET sub-sectors declined by 12% and 8% respectively.

Source: Statistics on post-school education in South Africa 2022. DHET, Pretoria 2024.

2

STRATEGIC PLAN 2022 - 2027

In 2024, HIGHER HEALTH reached the mid-point of the period covered by its current strategic plan. The goals of the plan, are as follows:

Goal 1

Through health awareness-creation, improve the knowledge of students in order to increase health-seeking behaviour which may contribute to timely completion of their studies.

Goal 2

Provide capacity building and skills training on health, wellness and psychosocial policy guidelines for staff at HIGHER HEALTH and PSET institutions.

Goal 3

Provide accessible, quality health and psychosocial support services to students across the PSET sector.

Goal 4

Expand HIGHER HEALTH 's teaching and learning through extramural Co-Curriculum courses that build health and wellness knowledge and skills of students.

Goal 5

Contribute to the development of an institutional environment that respects human rights and promotes responsible citizenship as a foundation for student health, wellness and psychosocial development.

Goal 6

Maintain HIGHER HEALTH 's funding base through a sustainable revenue model.

Goal 7

Strengthen and expand partnerships with institutions in all PSET sub-sectors, relevant government departments, funding and donor agencies, and other stakeholders in the broad field of student health, wellness and psychosocial development.

Goal 8

Promote the use of social and traditional media and digital communication to create awareness of the social and structural determinants of health.

3

HEALTH FOCUS AREAS

Higher Health focuses its strategic interventions on areas of health that are relevant to young people. These are outlined briefly below.

Mental health among young people has become a major public health priority worldwide and there is evidence that a high proportion of students experience psychosocial problems at some point. HIGHER HEALTH strives to create awareness of this and has expanded access to psychosocial services.

Gender-based violence is unacceptably high across South Africa and young women bear the brunt of this. Across the PSET sector women students are in the majority and HIGHER HEALTH assists institutions

to nurture gender equity, develop preventive interventions and provide access to comprehensive support for victims of GBV.

Sexual and reproductive health (SRH) is a major area of interest to youth and young adults. In partnership with other healthcare providers, HIGHER HEALTH strives to uphold the right of students to SRH products and services, including contraception, pregnancy testing, antenatal care and termination services, and early detection of certain cancers.

HIV, tuberculosis (TB) and sexually transmitted infections (STIs) remain priority epidemics in South Africa and young people are at the heart of the

national response. HIGHER HEALTH was established more than 25 years ago to reduce the vulnerability of students to HIV, and this remains a major focus area. HIGHER HEALTH seeks to contribute to national HIV, TB and STI initiatives, including the recently launched Close the Gap campaign of the Department of Health, which aims to boost uptake of HIV treatment.

Healthcare for students with special needs focuses on LGBTQI+ students and those living with disabilities. The HIGHER HEALTH approach centres on creating awareness of special health needs, promoting the right to appropriate and respectful care, and sensitising health workers.

Harmful use of alcohol and drugs is prevalent among students. We raise awareness of the risks, attempt to reduce the stigma which inhibits help-seeking, and link students to specialised services.

In recent years, HIGHER HEALTH has paid more attention to healthy lifestyles knowing that habits acquired early in life determine the later risk of acquiring non-communicable diseases (NCDs) that are beginning to overwhelm health systems globally.

4

THE HIGHER HEALTH MODEL

HIGHER HEALTH's support to students on university and college campuses is guided by a model that comprises three levels of intervention: knowledge transfer, practical learning, and linking students to health and psychosocial services.

Level 1

KNOWLEDGE TRANSFER: Sharing information on health and healthy living, within a supportive environment, is the first step to enabling students to take care of their wellbeing. Knowledge transfer encompasses awareness-raising campaigns and activities undertaken by student peer educators, more structured health education modules delivered through Co-Curriculum sessions, and – since 2024 – the accredited Civic and Health Peer Education Programme.

Level 2

PRACTICAL LEARNING. This involves students applying their new understanding of an aspect of health. They might take action to secure their own health by means of a self-administered assessment which indicates whether they are at risk of illness or harm and enables them to seek professional help. Students can also volunteer as peer educators to build health awareness and a supportive campus environment.

LINKING STUDENTS TO HEALTH AND PSYCHOSOCIAL SERVICES. These services include the toll-free crisis hotline for psychosocial support, more extensive professional counselling, and clinical services in the areas of HIV and TB prevention and treatment, SRH, NCDs, GBV and substance abuse. Some services are offered by campus health clinics and HIGHER HEALTH's mobile clinics, while others are accessed by referral to community health services run by NGOs and government.

The HIGHER HEALTH Model is underpinned by:

- The development of sector-wide policies and guidelines designed to contribute to a health-promoting climate at every PSET institution and extensive training for institutional staff on the implementation of these policies and guidelines.
- Our soft skills programmes which take both knowledge transfer and practical learning to a higher level and develop the link between the acquisition of soft skills and success in the academic field and the workplace. These programmes have been described in the CEO's Overview.

5

STRUCTURE OF HIGHER HEALTH

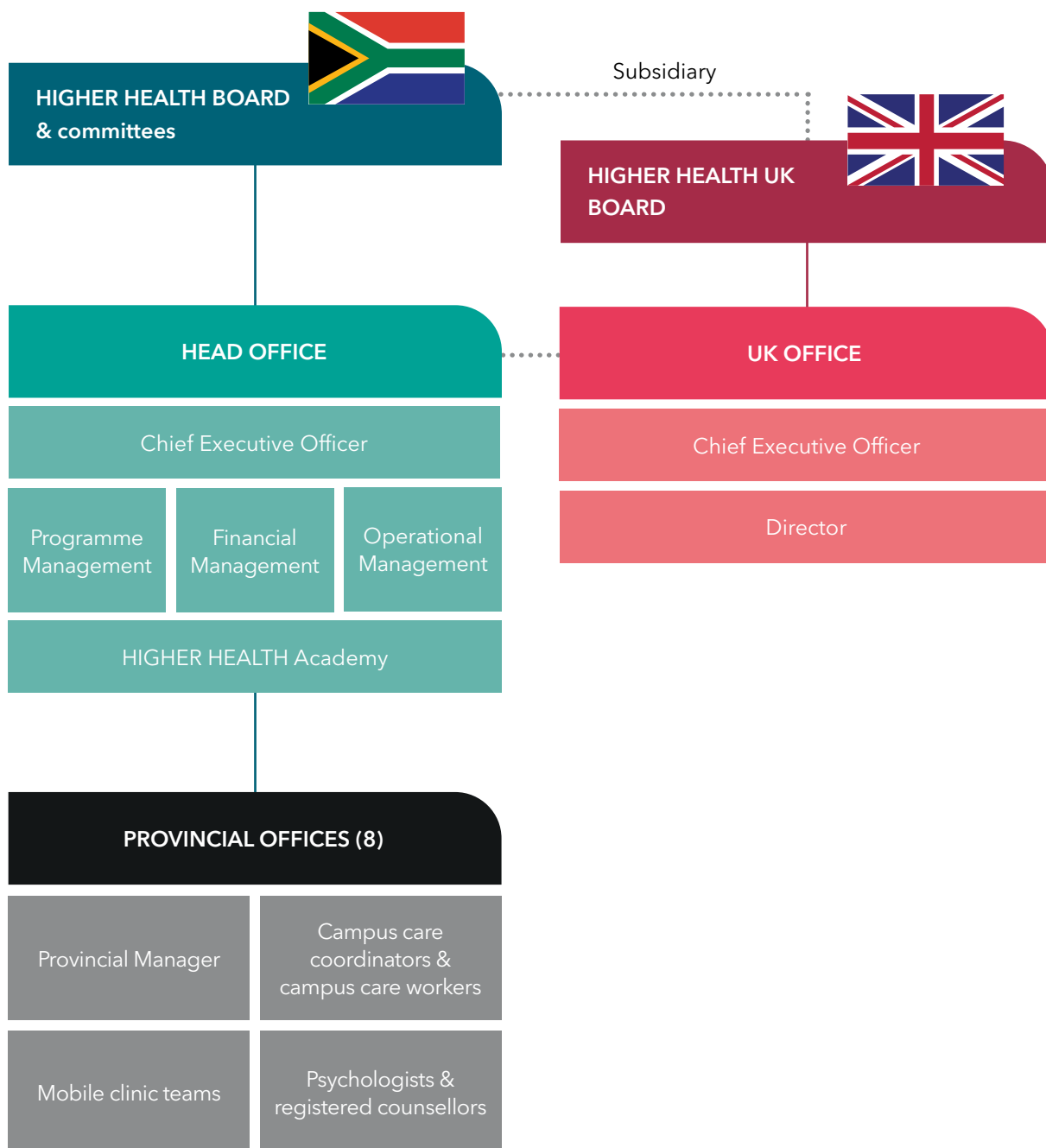
HIGHER HEALTH initiated restructuring in 2024/25 and the result is a less complex structure with professional health expertise built into the head office management team. Regional offices were phased out and eight provincial offices account directly to head office. The Northern Cape and Free State are served by a combined provincial office.

The programme management team at head office has been strengthened by the addition of a senior clinical psychologist and senior professional nurse who oversee psychosocial and clinical services delivered to campuses. The HIGHER HEALTH Academy is headed by the Director of E-Learning and Technology who is supported by specialists in

information technology, curriculum development and instructional design.

The other major development this year was the birth of a sister organisation, HIGHER HEALTH UK. This is a non-profit company registered in Britain and its focus is to equip young people with essential life skills for health, wellbeing and real-world readiness. HIGHER HEALTH UK is presently dedicated to the introduction of the Skills 4 Living programme. (See page 14.) It is governed by a separate board of directors nominated by the HIGHER HEALTH Board. Prof Ahluwalia is CEO of both organisations and there is strong collaboration between the South African and UK teams.

HIGHER HEALTH ORGANISATIONAL STRUCTURE





SECTION 3

DELIVERY OF CORE PROGRAMMES

1 INTRODUCTION

There was strong student participation in HIGHER HEALTH's well-established programmes in 2024, with improved performance in several areas and the university sub-sector delivering exceptional performance.

However, output was uneven with peaks in some activities and troughs in others.

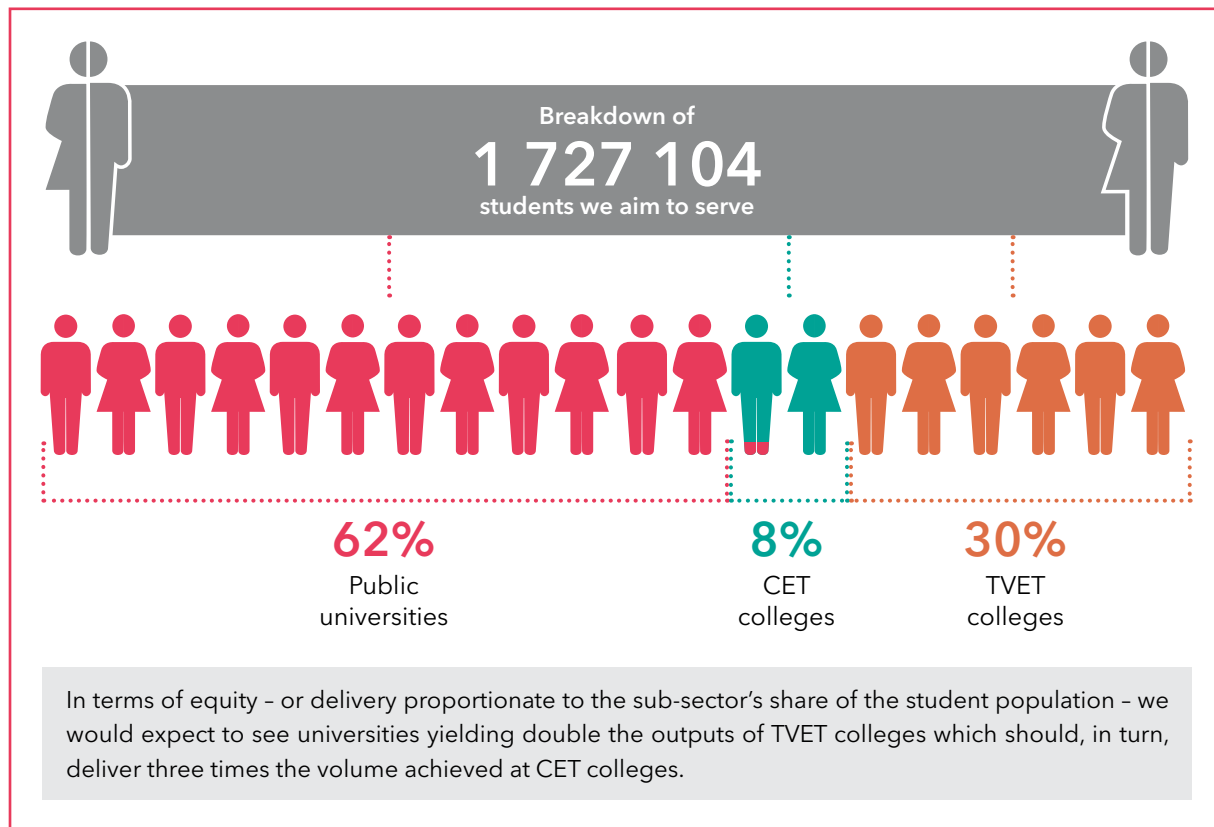
While the university sub-sector's figures were up across the board, the TVET sub-sector delivered a mixed performance, due mainly to the cyclical nature of our funding and resulting fluctuations in the number of employees we can deploy to campuses. Activity in the CET sub-sector, our newest area of work, was disappointing and our approach in this area requires some reconsideration.

The key to stabilising and reviving our TVET and CET college activities is a new grant from the National

Skills Fund (NSF) which enabled us to employ 75 young social work graduates who were unable to find employment in conventional social development organisations. They joined us in July 2024 as campus care workers and coordinators and we are thrilled to have them on board.

An overall increase was noted in students completing self-assessments of various health risks, using the tools developed by HIGHER HEALTH. However, overall participation in CoCurriculum sessions, which are designed to build students' knowledge of health and wellness, was lower for most health topics. While the university sub-sector returned stronger figures in 2024 than the previous year, these could not off-set the decline in the other sub-sectors.

However, our Civic and Health Peer Education Programme, which overlaps with the CoCurriculum in terms of content, went mainstream in the latter half



of 2024. This is an accredited learning programme that leads to a QCTO NQF Level 5 certificate. It builds a deeper understanding of a range of health, social and civic issues and develops soft skills that are valuable in academic life as well as the workplace. As this accredited programme gains strength it is likely that participation in the CoCurriculum may decline. In 2024/25, more than 7 400 students passed the final assessment for the Civic and Health Peer Education Programme.

Generally, screening for symptoms of communicable and non-communicable diseases (NCDs) showed a positive trend, led strongly by the universities. HIV testing numbers were slightly improved overall, and greatly so at universities. The sharp decline in HIV testing at TVET colleges and CET colleges, combined with their limited distribution of condoms, is a matter of some concern.

The uptake of sexual and reproductive health (SRH) services was more-or-less stable. This largely clinical area of programming has proved difficult to expand because it depends on health professionals with specific skills and knowledge.

Mental health services expanded substantially during 2024, due mainly to an immense effort by HIGHER HEALTH's clinical psychologists and registered counsellors. The team is now headed by a senior psychologist based at head office and this has built cohesion and increased service capacity.

The statistics in this section of the report are mainly for the academic year January to December 2024. This is because most of our programmes are planned and implemented to fit the academic calendar rather than the financial year.

2 CROSS-CUTTING TRAINING AND PEER EDUCATION ACTIVITIES

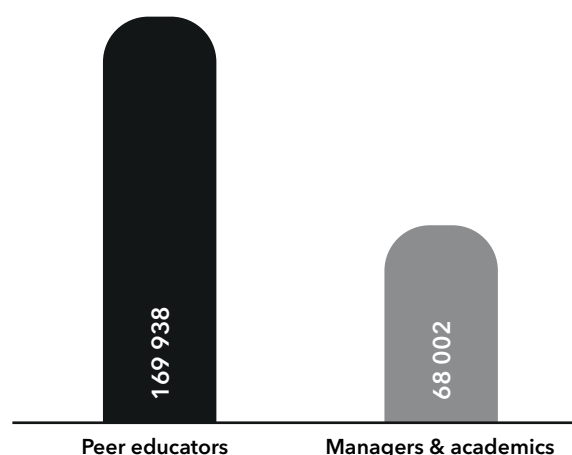
HIGHER HEALTH's health and wellness interventions rest on a partnership with individual universities and colleges. This requires building capacity at institution level. In some instances, our training is aimed at on-the-ground delivery – as with peer educators – and in others we build capacity among management and academics so that they can oversee health and wellness activities and initiate appropriate policies to promote health on campus.

At universities, training is coordinated and hosted by the institutions, frequently with participation by HIGHER HEALTH. At TVET and CET colleges, training is conducted by HIGHER HEALTH, working with institutional management and staff.

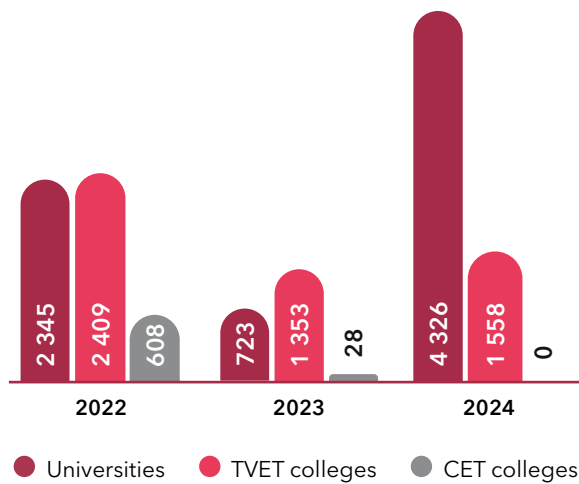
The number of peer educators trained in 2024 was 5 884, more than double the previous year. The university sub-sector saw an amazing sixfold increase in peer educators, setting the scene for an upsurge in programme delivery .

In 2024 a special focus was the induction of newly recruited social workers, training them on the HIGHER HEALTH model and equipping them to deliver the classroom-based aspects of the Civic and Health Peer Education Programme.

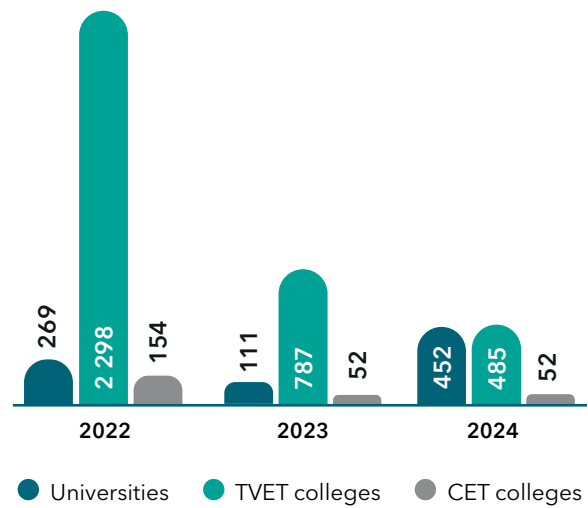
Overview of capacity development 2024



Trend in number of peer educators trained by sub-sector



Trend in number of health and wellness days



Historically, health and wellness days have played a vital role in promoting health, combining health awareness activities with screenings for communicable and non-communicable diseases, HIV testing and provision of contraceptives. HIGHER HEALTH's flagship campaign, First Things First, plays a similar role particularly during orientation weeks at the start of the academic year.

Over the last five years or so, there has been a gradual decline in health and wellness days, partly due to the disruptive COVID-19 pandemic and partly because of the development of the CoCurriculum and the Civics and Health course. In 2024 we recorded 962 health and wellness days (2023: 950) and 659 First Things First campaigns, with notable participation from TVET colleges.



HIGHER HEALTH marked some significant milestones in the delivery of its soft skills programmes, formal modular learning options that combine health education, life skills, soft workplace skills, and the promotion of civic values in a single package.

The first curriculum was accredited by the Quality Council for Trades and Occupations (QCTO) in 2022. Known as the Civic and Health Peer Education Programme, it underwent pilot testing in 2023 and 2024. Feedback from stakeholders prompted the addition of two modules to the six comprising the original curriculum. This expanded qualification, known as the Civic and Soft Skills Programme, was also accredited by the QCTO. Students who complete all modules in both programmes and pass a final assessment achieve a recognised NQF level 5 skills certificate. However, the six-module programme carries 30 study credits, while the eight-module programme carries 60 credits.

SET UP OF HIGHER HEALTH ACADEMY

The year saw the physical establishment of the HIGHER HEALTH Academy, the dedicated training facility accredited by the QCTO to deliver the CSSP. The position of Director of E-Learning and Information technology – effectively, head of the Academy – was created but has still to be filled on a permanent basis. The Academy remains accountable to the Chief Executive Officer and the HIGHER HEALTH Board.

LESSONS FROM THE PILOT PROJECT

The pilot of the Civic and Health Peer Education Programme, which attracted 1 531 active participants, concluded in mid-2024. It confirmed that the course content was on the mark but sounded a clear warning that an all-digital method of delivery was inappropriate for most students. Only nine participants completed all modules while a substantially larger number did a few modules but could not conclude the full programme by the end of June.

Connectivity, access to computers and digital skills vary across the student population and the programme needs to accommodate this. Consequently, HIGHER HEALTH decided that two programme offerings were necessary:

- An online curriculum for students with good connectivity who require minimal support to engage online and complete the course.
- A hybrid curriculum, envisaged mainly for TVET and CET college students, which would combine online elements with classroom presentations and discussions facilitated by a HIGHER HEALTH campus care worker/coordinator.

RESOURCING FOR MAINSTREAM DELIVERY

For some time HIGHER HEALTH has been planning to incorporate the untapped resource of qualified and jobless social workers into its campus activities. We felt there was a real synergy between social work training and our need for campus workers who were multiskilled – able to undertake individual psychosocial counselling, facilitate student growth through group work, and coordinate campus-wide health activations. We had been discussing with the NSF possible funding for supervised entry-level jobs for unemployed social workers in order to enhance their skills and improve their employment prospects.

When we added facilitation of the hybrid Civic and Health Peer Education Programme to the proposed role of social workers, we offered the NSF much greater value. Successful delivery of this programme would add vital lifeskills and soft workskills to the college and university experience, producing more capable graduates in a wide variety of disciplines. This dual benefit became the basis of a three-year funding agreement between the NSF and HIGHER HEALTH for an amount of R250 million. This generous amount will provide financial stability and the required human resource base for this programme as it moves through its developmental years.

ACTIVATING THE HYBRID MODEL

Once the team of social workers had been appointed and undergone training, the Civic and Health Peer Education Programme hybrid model was offered twice: in the last quarter of 2024 and the first quarter of 2025. The 2024 results were encouraging but still below target, while the first run of 2025 delivered strong course completion.

The relevant figures are presented below. What they indicate is that HIGHER HEALTH is on course to meet the terms of its agreement with the NSF and ensure that 30 000 students enjoy the benefits of the full programme over the next three years.

In March 2025, the DHET incorporated the eight-module Civic and Soft Skills Programme (CSSP) into the NATED Report 191 National N Diploma qualification which is widely offered by TVET colleges. Our programme will comprise part of the practical requirement for the engineering and business diplomas, adding a "soft skill" dimension to

the technical skills that they develop in the remaining practical hours.

The past year has been a critical year for the development of students' soft skills. Our strong belief in the inherent value of the course to young people kept us going when the practical problems of implementation seemed overwhelming. We believe we have found the route to high rates of programme completion and we are grateful to those who helped us chart this direction and secure the necessary resources.

Our confidence has also been boosted by the partnerships HIGHER HEALTH has forged with universities and colleges in the Greater Manchester area of the United Kingdom in order to conduct of a pilot run of our soft skills curriculum at their institutions. Our sister organisation, HIGHER HEALTH UK, is responsible for the management of this process. A brief description of the UK partnership appears in the CEO's Overview on page 14.

Overview of Civic and Health Peer Education Programme

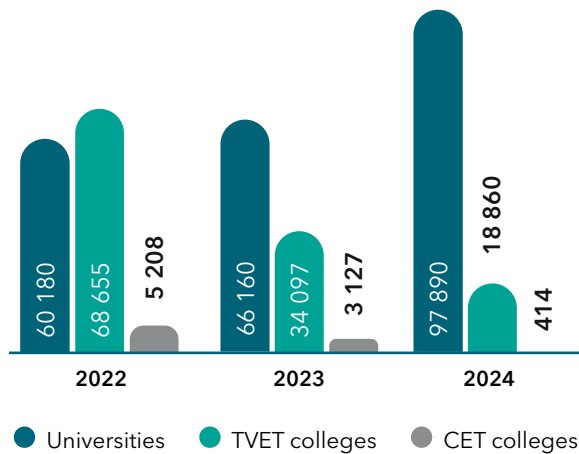
	Pilot 2023 Online only	2024 Hybrid platform	2025 Hybrid platform
 Number of active participants*	1 531	2 803	11 277
 Number of final assessment candidates	9	783	6 919
 Number who passed final assessment	9	701	6805

**As with many online learning programmes, a large number who enrol do not pursue the course. These have been excluded from the above table.*

4

SEXUAL AND REPRODUCTIVE HEALTH INCLUDING HIV, TB AND STIS

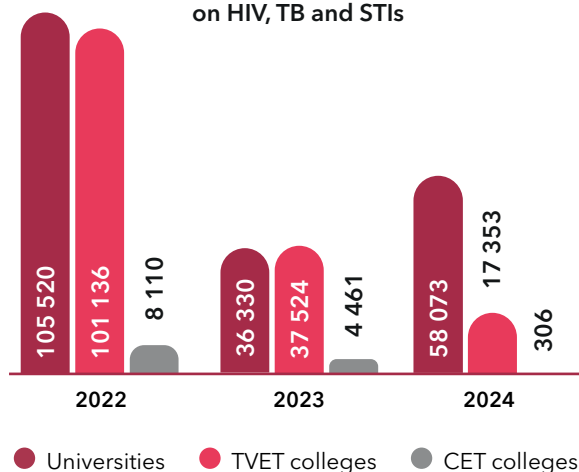
Self-assessment of HIV, TB, STI and SRH risk



HIGHER HEALTH takes a rights-based and comprehensive approach to sexual and reproductive health (SRH). This incorporates HIV, as many behavioural and social factors that impact on SRH also influence the risk of acquiring HIV.

Participation in Co-Curriculum sessions and performance of health risk assessments has been quite variable in recent years in respect of SRH, HIV, TB and STI. While the university sub-sector performed relatively well in 2024, participation in the TVET sub-sector was significantly lower than in previous years, and the CET colleges were hardly active. Even when one considers mobile clinics see a substantial number of TVET college students, the figures for this sub-sector are lower than in previous years.

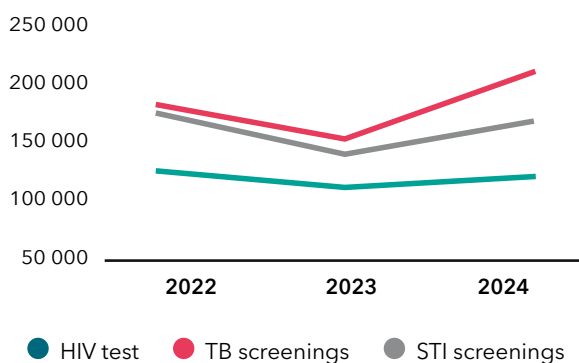
Attendance at Co-Curriculum sessions on HIV, TB and STIs



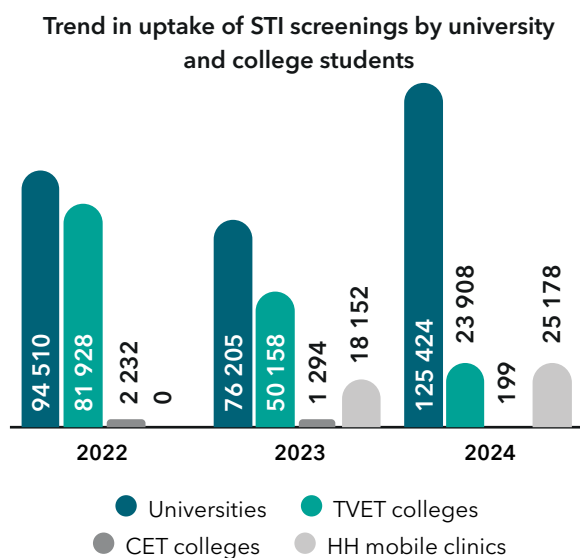
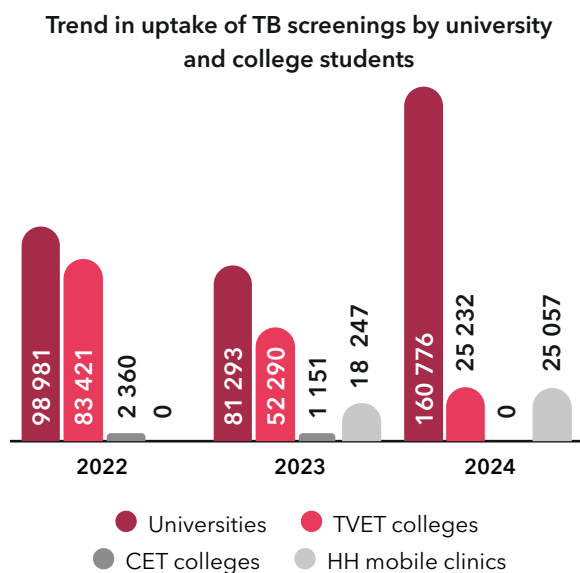
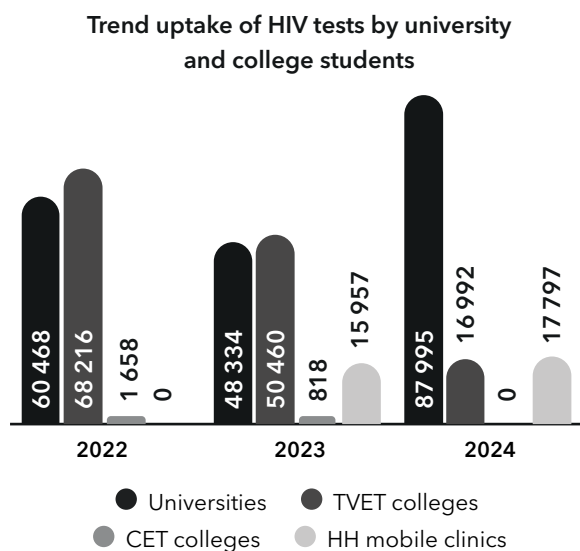
HIGHER HEALTH is alive to the fact that we cannot lower our guard against HIV. According to the Tembisa 4.8 model, South Africa has about 178 000 new infections annually and young people between 15 and 24 years are disproportionately affected by HIV. Due largely to gender inequality, the highest risk of HIV infection is to young women. The fact that 22.5% of HIV-positive individuals nationally are not on treatment contributes to the persistently high risk of infection.

The decline in participation in HIV, TB and STI prevention activities at TVET colleges was mainly attributable to the changeover in grant funding. During the gap between grants, lasting several months, HIGHER HEALTH was unable to deploy campus care workers to TVET and CET colleges. Furthermore, when a new intake of campus care workers came on board in July/August 2024, their priority was launching the hybrid model of the Civic and Health programme.

Trend in HIV testing and screening for TB and STIs



In terms of the availability of HIV prevention products on campuses, 2024 once again presented a mixed picture, as the graphic on pages 34 and 35 indicates.

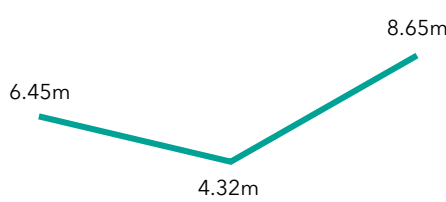
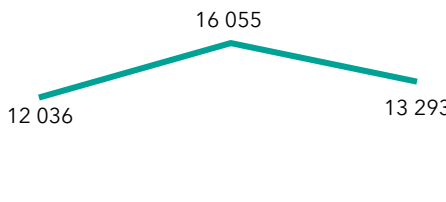


Condom distribution remains a cornerstone of our SRH programme because it protects against HIV, other STIs and unintended pregnancy. The major recovery in condom supplies and distribution in 2024 was impressive but almost entirely due to the effort of universities. TVET colleges, CET colleges and mobile clinics accounted for a mere 6% of the 8.65 million condoms made available to the PSET sector. HIGHER HEALTH accessed unusually large stocks of female condoms, amounting to 28% of the total, which enabled woman students to take control of their protection.

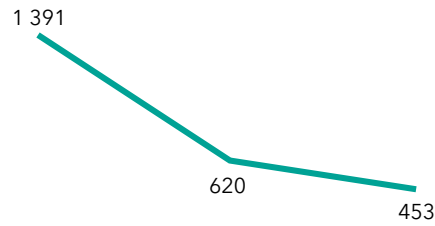
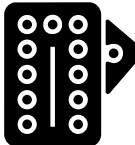
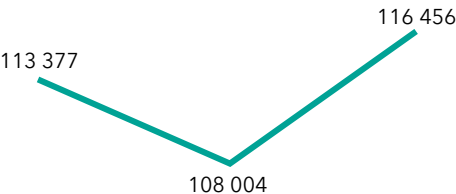
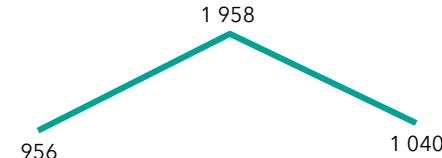
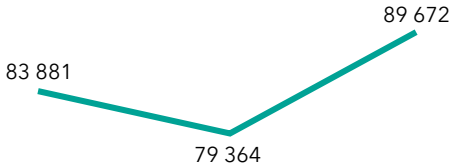
Oral PrEP – a daily antiretroviral pill that offers high protection against HIV infection – was seen as a potential breakthrough in improving HIV prevention

among young women. However, uptake of oral PrEP has been moderate rather than dramatic. Even South Africa, which leads the world, has only recorded 1.34 million PrEP initiations since the product was introduced in 2016. In the PSET sector demand for PrEP has been largely stable over the last few years. It is available on-site at some institutions and by referral to community clinics run by NGOs and government. In 2024, universities accounted for the bulk of PrEP provision.

The downward trajectory in providing antiretroviral treatment (ART) to students who have tested positive for HIV is disappointing. The 2024 rate of 46% in the PSET sector is well below the national figure of 81.5% which, in turn, falls short of the global target of 95%.

Sexual and reproductive health services: 2024 performance and three-year trend												
Figures for 2024	Universities	TVET colleges	HH mobile clinics	Three-year trend in delivery								
Condoms distributed 8 646 938 	94%	4%	2%	<table><thead><tr><th>Year</th><th>Condoms distributed (m)</th></tr></thead><tbody><tr><td>2022</td><td>6.45</td></tr><tr><td>2023</td><td>4.32</td></tr><tr><td>2024</td><td>8.65</td></tr></tbody></table>	Year	Condoms distributed (m)	2022	6.45	2023	4.32	2024	8.65
Year	Condoms distributed (m)											
2022	6.45											
2023	4.32											
2024	8.65											
PrEP accessed 13 293 	80%	14%	6%	<table><thead><tr><th>Year</th><th>PrEP accessed</th></tr></thead><tbody><tr><td>2022</td><td>12 036</td></tr><tr><td>2023</td><td>16 055</td></tr><tr><td>2024</td><td>13 293</td></tr></tbody></table>	Year	PrEP accessed	2022	12 036	2023	16 055	2024	13 293
Year	PrEP accessed											
2022	12 036											
2023	16 055											
2024	13 293											

Sexual and reproductive health services: 2024 performance and three-year trend

Figures for 2024	Universities	TVET colleges	HH mobile clinics	Three-year trend in delivery								
Linked to ART 453 	88%	8%	3%	 <table><tr><th>Year</th><th>Value</th></tr><tr><td>2022</td><td>1 391</td></tr><tr><td>2023</td><td>620</td></tr><tr><td>2024</td><td>453</td></tr></table>	Year	Value	2022	1 391	2023	620	2024	453
Year	Value											
2022	1 391											
2023	620											
2024	453											
Contraceptives accessed 116 456 	74%	10%	16%	 <table><tr><th>Year</th><th>Value</th></tr><tr><td>2022</td><td>113 377</td></tr><tr><td>2023</td><td>108 004</td></tr><tr><td>2024</td><td>116 456</td></tr></table>	Year	Value	2022	113 377	2023	108 004	2024	116 456
Year	Value											
2022	113 377											
2023	108 004											
2024	116 456											
Linked to termination of pregnancy services (TOP) 1 040 	71%	1%	28%	 <table><tr><th>Year</th><th>Value</th></tr><tr><td>2022</td><td>956</td></tr><tr><td>2023</td><td>1 958</td></tr><tr><td>2024</td><td>1 040</td></tr></table>	Year	Value	2022	956	2023	1 958	2024	1 040
Year	Value											
2022	956											
2023	1 958											
2024	1 040											
Cancer screenings 89 672 	31%	22%	47%	 <table><tr><th>Year</th><th>Value</th></tr><tr><td>2022</td><td>83 881</td></tr><tr><td>2023</td><td>79 364</td></tr><tr><td>2024</td><td>89 672</td></tr></table>	Year	Value	2022	83 881	2023	79 364	2024	89 672
Year	Value											
2022	83 881											
2023	79 364											
2024	89 672											

HIV testing and the route to treatment

	HIV test	Self-screen	Total	
Tested	122 973	1 945	124 918	HIV tested: 0.79% positive
Tested positive	900	90	990	Self-screened: 4.6% positive
Referred for care	-	-	614	62% who tested positive referred
Initiated on ART	-	-	453	46% who tested positive started ART

In relation to TB, 1.6% of those screened were symptom-positive and nearly all were referred for further investigation. Our data do not reflect whether they ultimately tested positive for TB and began treatment.

A total of 12 174 students received treatment for an STI. About half of these were referred from screening services and others came directly to mobile or campus clinics. Nearly 94% of those treated attended universities. Student health services attempt to follow the established practice of treating the partner of the primary patient, but this remains challenging due largely to stigma and confidentiality concerns.

There has been little growth in the number of contraceptives supplied to students in recent years. Given that young women constitute most students, the 2024 figure of 116 456 contraceptives supplied seems to be inadequate to meet the need. However, it is possible that relatively easy access to condoms, the ability of condoms to protect against HIV and STIs, and the absence of side-effects makes condoms the contraceptive of choice for many students. There are supply-side constraints in respect of other contraceptives. Only some students have reliable access to them through campus or mobile clinics, and the individual's preferred contraceptive may not always be in stock.

Our mobile clinics are taking the lead in screening for cervical, breast, prostate and penile cancer. Not only did they perform 47% of all screenings

done in 2024, but they made 86% of referrals for further investigation of possible malignancy. In the case of cervical cancer, 3.5% of students screened – amounting to 1 289 students – required further investigation and possible intervention. Since cervical cancer can be cured if caught early, but is often fatal if discovered late, this service is extremely valuable.



Global Fund programme concludes

The three-year Adolescent and Young People's (AYP) Programme, supported by the Global Fund to Fight AIDS, TB and STIs, has had a strong focus on woman students and it drew to a close in March 2025.

The AYP was available to students at 25 campuses of 16 TVET college. Its objectives were to:

- Reduce new HIV infections among young people.
- Reduce teenage and unwanted pregnancies.
- Reduce GBV.
- Increase retention of students in PSET.
- Improve economic opportunities for young people.

The programme aimed to offer each student who enrolled in it the benefit of a range of health products and interventions related to the above objectives. HIGHER HEALTH's performance areas were HIV testing, delivery of HIV prevention interventions, and ensuring students who tested positive for HIV were linked to ART.

Both the number of students assisted by the AYP and HIGHER HEALTH's performance against programme targets improved over the grant period. In 2024/25 a total of 31 198 HIV tests were performed and 41 323 HIV prevention interventions.

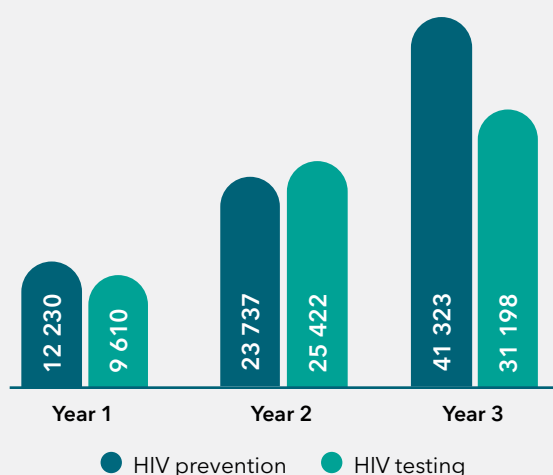
The year's performance was commendable, especially as HIGHER HEALTH only has access to students for eight months a year.

The AYP made health and hygiene products available free of charge to participating students by means of 37 vending machines accessed by a pin number. Stock control data in 2024 indicated the highest demand was for sanitary pads and pregnancy tests – well ahead of uptake of male and female condoms. This suggested the vending machines might not be supporting HIV prevention objectives as much as intended. HIGHER HEALTH has since introduced digital product "tutorials" that students can access on their mobile phones by scanning QR codes posted on vending machines.

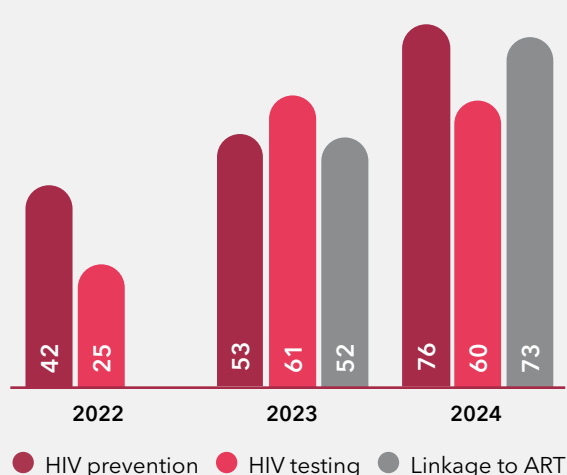
The AYP grant was managed by three Global Fund principal recipients responsible for different provinces: the Networking AIDS Coalition of South Africa (NACOSA), the AIDS Foundation of South Africa (AFSA) and Beyond Zero. We thank them for their support and guidance.

HIGHER HEALTH was included in the AYP grant extension period from April to September 2025, and we are hopeful we will be selected for the next multi-year grant.

Uptake of AYP services across grant period



Percent of targeted number of students reached



Note: 14 HIV-positive students were linked to ART in year 2 and 61 were linked in year 3.

NON-COMMUNICABLE DISEASES

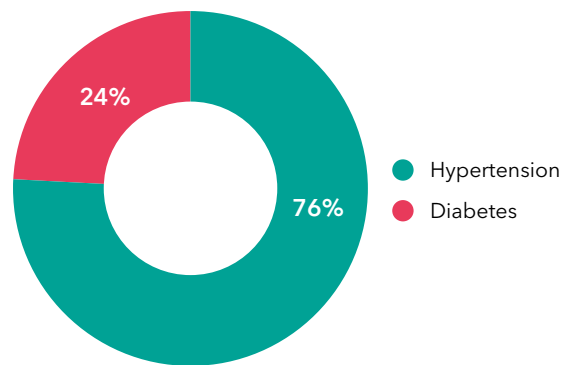
For decades, non-communicable diseases (NCDs) formed a slowly gathering storm in South Africa, as an increasing slice of the population was absorbed into the modern lifestyle of long commutes, stressful work, highly processed foods, and little exercise. The NCD storm has now broken: cardiovascular diseases and diabetes currently rank among the country's top causes of ill-health and death.

Typically these diseases are not diagnosed in young people, but the lifestyles which cause them are shaped early in life. HIGHER HEALTH adopts a dual approach:

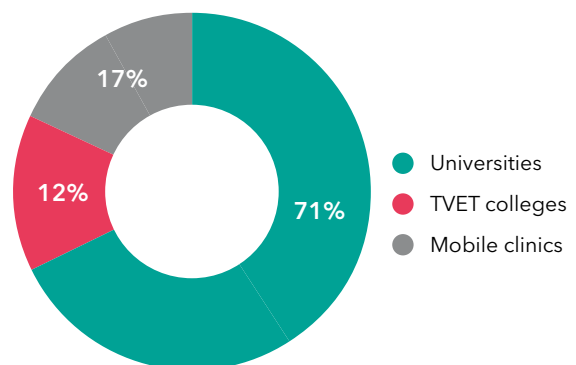
- Raising awareness of NCD risk factors and encouraging healthy lifestyles.
- Screening to identify the relatively small number of students who develop NCDs at a young age and referring them for further investigation and management.

In 2024, a total of 227 108 screenings for hypertension and diabetes were performed. In addition, TVET colleges and mobile clinics screened 12 184 individuals for anaemia.

Types of NCD screening 2024



Sites of NCD screening 2024



GENDER-BASED VIOLENCE PROGRAMME

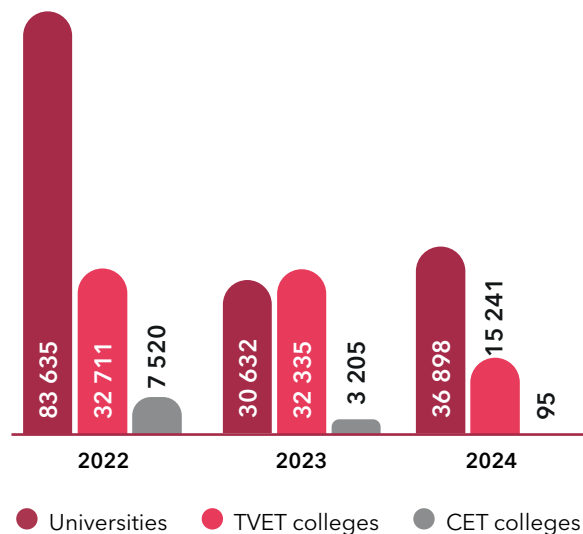
Gender-based violence (GBV) continues to be recognised as a national epidemic, predominantly affecting women, and young women in particular. The demographic risk profile fits the majority of students in the PSET sector. Where women students have left the parental home to study at a distant institution, living independently for the first time, their vulnerability is likely to be heightened.

HIGHER HEALTH is alive to the potential for GBV in PSET settings and puts a high premium on preventive interventions. These include:

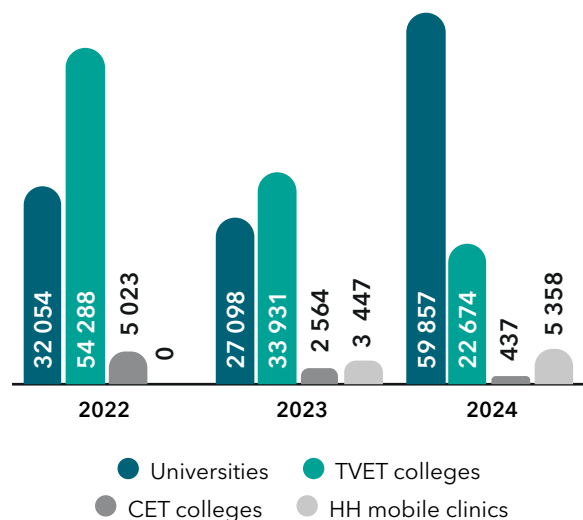
- Creating awareness of the nature of GBV among students and staff and reducing stigma surrounding those subjected to it. In 2024, CoCurriculum sessions on GBV reached 52 234 students (2023: 66 172).
- Developing policy frameworks and guidelines that promote gender equity on campuses and censure discrimination or any form of gender-based abuse. Enabling institutions to adopt and implement these is a significant area of our work.
- Facilitating campus events for young men under the UNESCO-inspired Transforming MENTalities programme which focuses on the social and cultural factors that impact on GBV. While the themes of these events differed according to the particular needs of universities and colleges, the unifying purpose was to provide a safe space for young men to explore psychosocial issues. At least 10 events were held in 2024/25, with an average attendance of 100 per event.
- Increasing recognition of personal GBV risk. This is facilitated by students completing a self-assessment questionnaire, an activity that 88 326 opted for in 2024 (2023: 67 031).
- Referring those at risk of perpetrating or suffering GBV to dedicated counselling services. The referral may occur after a student self-assessment, through the Crisis Helpline, or when a student visits another health service and the health worker is alert to an underlying problem.

GBV victims are generally referred to Thuthuzela Centres in the community for comprehensive medical and psychosocial care and support for initiating criminal proceedings where they choose to do so.

Participation in CoCurriculum sessions on GBV



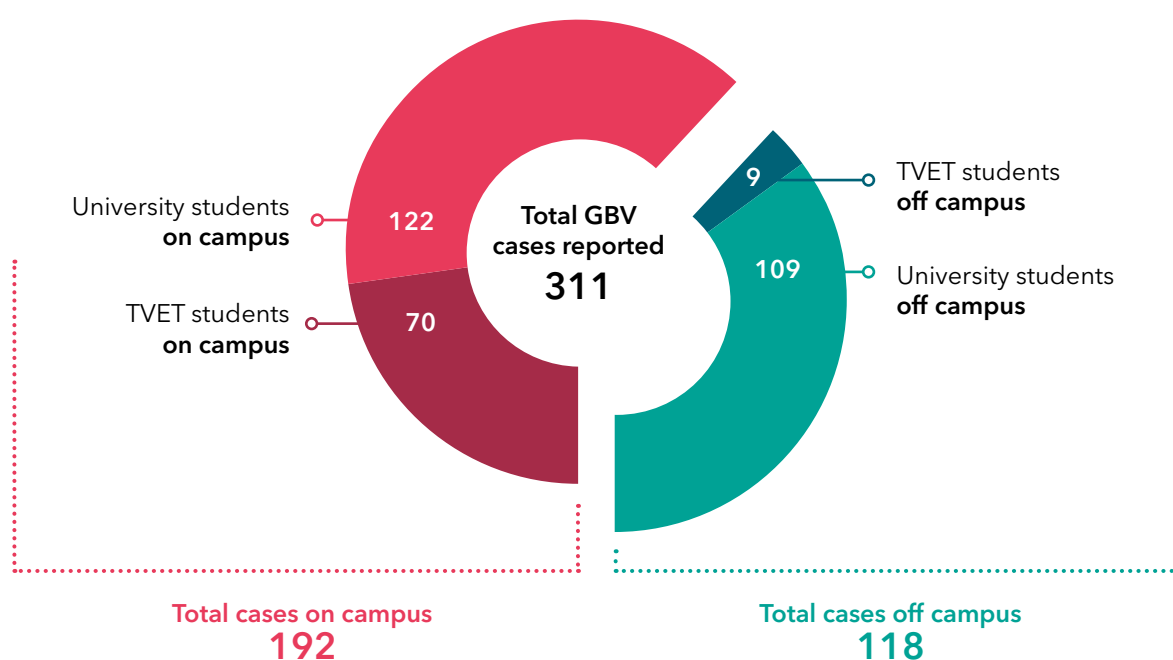
Students performing self-assessment of GBV risk



Uptake of GBV services in 2024

	Universi- ties	TVET colleges	CET colleges	Mobile clinics	Total
Received on-site GBV counselling	544	144	0	1	689
Received off-site GBV counselling	41	13	0	0	54

Number of students reporting GBV on and off campus in 2024



A total of 311 GBV cases were reported by students in 2024, 32% more than in the previous year. Nearly two out of three of these occurred on the campuses of universities and TVET colleges, indicating that - despite our best efforts - our institutions are still not safe spaces.

While almost all instances at TVET colleges occurred on campus, at universities off-campus incidents of GBV were reported almost as frequently as those on campus. The precise risks that expose university students to off-campus GBV are not understood but may relate to their accommodation arrangements away from their families.

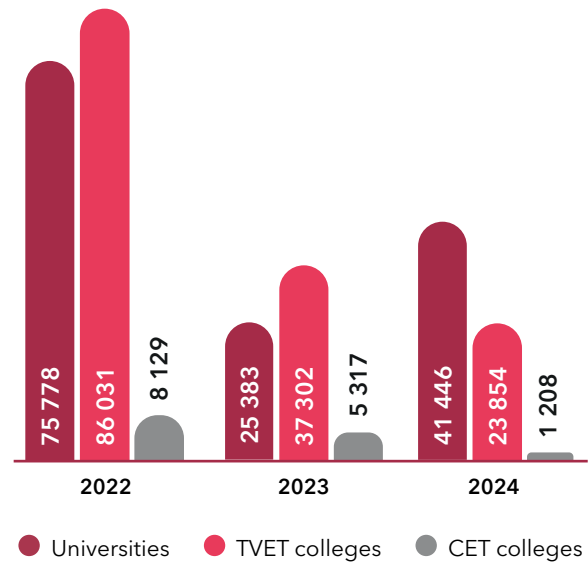
Many mental disorders first appear during adolescence and early adulthood and therefore mental health should form part of any youth health programme. Furthermore, young people at universities and colleges are subject to unique stressors that increase their need for psychosocial support. A national survey of 29 000 university students, conducted by USAf and the South African Medical Research Council (SAMRC) and published in 2022, found that one in five students would benefit from psychosocial support and a small sub-group required specialised clinical care.

HIGHER HEALTH recognised this need some years ago and initiated campaigns to raise awareness of mental health and combat the stigma that often deters individuals from seeking care. We also offer CoCurriculum sessions on mental health that were attended by 66 508 students in 2024 and a mental health self-screening tool, used by 102 766 students in the reporting period.

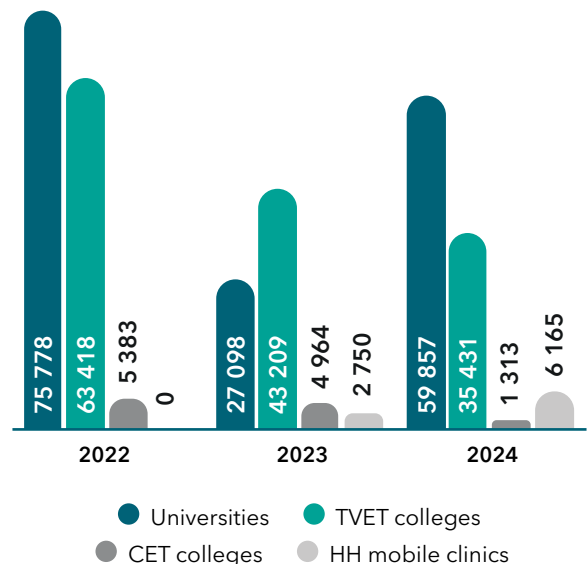
Over the last five years, HIGHER HEALTH has expanded students' options for psychosocial care. Currently these include campus clinics and counselling services, the 24-hour toll-free helpline run in partnership with the South African Depression and Anxiety Group (SADAG), and a team of HIGHER HEALTH psychologists and registered counsellors.

Our in-house mental health team takes referrals from the helpline - which offers a focused crisis intervention service - and can be accessed at our mobile clinics that service campuses on a roster basis. They also play an invaluable role training frontline campus staff - for example, student residence managers - to deal with psychosocial crises and refer students for further counselling.

Students attending CoCurriculum sessions on mental health



Students performing self-assessment of mental health risk



Mental Health services provided 2024



Campus counselling
services

7 461
sessions



24-hour crisis
helpline

12 109
phone calls & messages



HH teams of
psychologists

31 993
sessions

Top reasons for students contacting the helpline or HIGHER HEALTH psychologists



Stress
60%



Information
13%



Depression
12%



Anxiety
12%

Data drawn from HIGHER HEALTH records indicate that stress is by far the most common mental health condition experienced by students. While academic stress tops the list, family and relationship issues, trauma and bereavement are also significant stress factors.

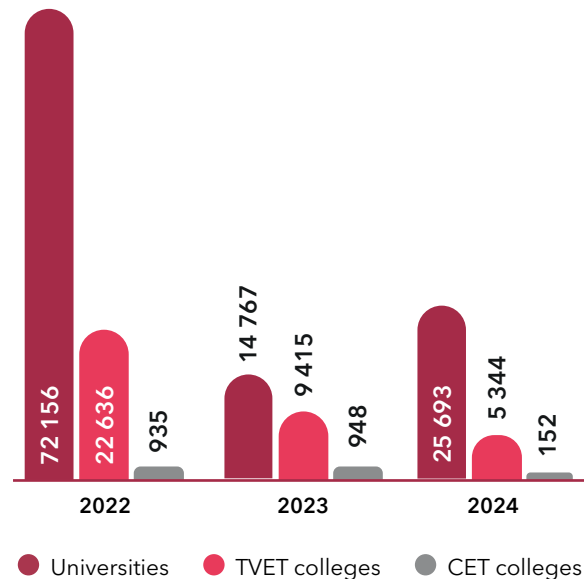
VULNERABLE POPULATIONS PROGRAMME

A substantial number of students belong to a group with special health needs and HIGHER HEALTH strives to improve their access to appropriate services. We focus on three such groups: the LGBTQI community, students living with disabilities, and students with a substance-use problem.

Stigma can be a major barrier to health-seeking by members of vulnerable populations and affect their psychosocial wellbeing. Our CoCurriculum sessions on gender, disability and substance use – which are geared to the general student population as well as members of vulnerable groups – deal with stigma within a human rights framework.

Our efforts to reach out to students with substance and alcohol use issues are beginning to bear fruit. In several provinces, we have partnered with liquor boards to raise awareness of unsafe alcohol consumption. More students are willing to use the self-assessment tool to gauge whether their alcohol and substance habits are risky. While only a small number were diagnosed with addiction by healthcare professionals, more than 300 required counselling for harmful patterns of use. Our service partner in most instances is the South African National Council on Alcoholism and Drug Dependence (SANCA).

Participation in CoCurriculum sessions: gender identity, disability and substance use



The German Agency for International Cooperation (GIZ) has provided funding for a one-year programme which leverages male peer educators to conduct awareness campaigns among male students and alcohol risk is one of the focal areas.

Students assisted through alcohol and substance use interventions

	Universi- ties	TVET colleges	CET colleges	Mobile clinics	Total
Self-assessment of alcohol/ substance use risk	15 707	3 467	141	26	19 341
Referral for professional help	3 313	96	0	26	3 435
Received counselling for alcohol/substance abuse	50	264	0	0	314
Diagnosed with alcohol or substance addiction	16	13	0	0	29

HIGHER HEALTH recognises the considerable resources that many universities possess and may utilise to enhance the health and wellbeing of students. We strive to work in partnership with these institutions to ensure that their resources are strategically invested and contribute to a national programme. Where universities have been recently established or historically disadvantaged, HIGHER HEALTH offers additional support to develop campus capacity for health and wellness.

In 2024, this arrangement was extraordinarily fruitful and the 26 public universities collectively raised their game. On most programme indicators, universities as a group performed significantly better than in 2023 and met their targets for 2024. Across the sub-sector, only delivery on CoCurriculum sessions, though somewhat improved, failed to achieve targets. Performance varied considerably among institutions.

Broadly speaking, HIGHER HEALTH's assistance to the university sub-sector centres on:

- Making financial grants for health and wellness activities and supporting institutions' management of these grants.
- Developing institutional human resource capacity for the planning and delivery of health interventions.
- Providing strategic and technical guidance on student health and wellness programming.
- Offering knowledge transfer platforms and services that are available to students across the PSET sector, such as the online soft skills programme and the toll-free helpline for psychosocial care.
- Coordinating across the sub-sector and engaging with relevant stakeholders.

The current HIGHER HEALTH grant cycle for universities commenced in June 2023 and concluded at the end of March 2025. Overall, by the end of the cycle only 35% of the total grant amount had been disbursed, due to delays in the availability of funding. The grant cycle has been extended into the new financial year and the balance of allocated grants will be disbursed during this period.



Summary of grant disbursements and achievement of targets as at 31 March 2025

University	Amount allocated for 2023/25	Amount disbursed by 31/3/25	Peer educator training	CoCurriculum attendance	Risk self-assessment	Linkage to service
Cape Peninsula Univ of Technology (CPUT)	R600 000	R310 000	●	●	●	●
Central University of Technology (CUT)	R500 000	R150 000	●	●	●	●
Durban University of Technology (DUT)	R650 000	R315 000	●	●	●	●
Mangosuthu University of Technology (MUT)	R400 000	R60 000	●	●	●	●
Nelson Mandela University (NMU)	R500 000	R370 000	●	●	●	●
North West University (NWU)	R500 000	R75 000	●	●	●	●
Rhodes University (RU)	R500 000	R75 000	●	●	●	●
Sefako Makgatho Health Sciences Univ (SMU)	R400 000	R120 000	●	●	●	●
Sol Plaatje University (SPU)	R400 000	R120 000	●	●	●	●
Stellenbosch University (SU)	R650 000	R315 000	●	●	●	●
Tshwane University of Technology (TUT)	R600 000	R180 000	●	●	●	●
University of Cape Town (UCT)	R650 000	R315 000	●	●	●	●
University of Fort Hare (UFH)	R600 000	R180 000	●	●	●	●
University of the Free State (UFS)	R650 000	R195 000	●	●	●	●
University of Johannesburg (UJ)	R600 000	R180 000	●	●	●	●
University of KwaZulu-Natal (UKZN)	R650 000	R315 000	●	●	●	●
University of Limpopo (UL)	R650 000	R195 000	●	●	●	●
University of Mpumalanga (UMP)	R400 000	R90 000	●	●	●	●
University of Pretoria (UP)	R450 000	R135 000	●	●	●	●
University of South Africa (Unisa)	R600 000	R180 000	●	●	●	●
University of Venda (Univen)	R650 000	R195 000	●	●	●	●
University of Zululand (Unizulu)	R500 000	R150 000	●	●	●	●
University of the Western Cape (UWC)	R500 000	R250 000	●	●	●	●
University of the Witwatersrand (Wits)	R600 000	R310 000	●	●	●	●
Vaal University of Technology (VUT)	R550 000	R182 500	●	●	●	●
Walter Sisulu University (WSU)	R450 000	R135 000	●	●	●	●
Total	R14 200 000	R4 997 500	●	●	●	●

● Target achieved ● Target not achieved

Two main factors drove this improved performance by the University Programme:

- Institutional Health and Wellness Committees (HICCs) functioned effectively at 20 universities, providing oversight, support and guidance to those responsible for implementing HIGHER HEALTH activities. The six universities that had no HICCs were in the process of constituting them in 2024 and we expect to see these up and running in 2025.
- Universities were encouraged to expand the pool of peer educators by drawing in volunteers from

other student support structures, such as mental health counselling services, gender units and transformation units. Peer educators are a driving force for HIGHER HEALTH at universities and the six-fold increase in their numbers (compared to 2023) was central to the surge in health and wellness output.

HIGHER HEALTH was also more visible on university campuses thanks to the display of our branding on educational and promotional materials.

9 TVET AND CET COLLEGES PROGRAMMES

TVET COLLEGE PROGRAMME

The TVET sub-sector comprises a greater number of institutions than the university sub-sector, and colleges are typically much smaller than universities and more dispersed across rural areas. They are also leaner institutions than universities, with fewer support services, and only a few have on-site clinic facilities to help implement HIGHER HEALTH programmes. For this reason, we have deployed operational staff to TVET colleges and invested in staffed mobile clinics to supplement the contribution of the colleges.

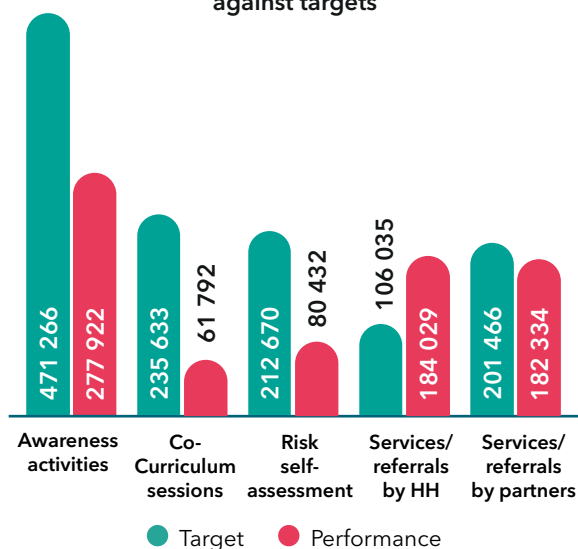
The performance of HIGHER HEALTH programmes at TVET colleges depends critically on the availability of these human resources, which in turn is linked to funding cycles. By the end of the 2023/24 financial year, the NSF grant which had enabled us to employ campus health and wellness coordinators for all colleges had come to an end. While the NSF approved a new grant which enabled us to employ and provide workplace learning opportunities for 75 social workers at TVET and CET colleges, these positions were only filled in July/August 2024.

Although HIGHER HEALTH staff trained more peer educators in 2024 than in the previous year, knowledge transfer activities attracted a significantly smaller number of students. Awareness campaigns, conducted through health days and activations, reached 59% of the targeted number of students. Attendance at CoCurriculum sessions totalled 61 792 or 26% of the target, and the number of self-assessments of health risks was 80 432, reaching 38% of the target.

The data on linkage of students to health services tell a much more positive story, due to the increase in the services offered by the HIGHER HEALTH mobile clinic fleet and reliable partnerships with NGOs. Provision of services and referrals involving our own staff exceeded the target considerably (173%) while service provision and referrals involving partners were just below target (90%).

During 2024 the HIGHER HEALTH clinical service expanded to include provision of some long-acting contraceptives and chronic medication.

TVET college sub-sector performance against targets



Memorandums of understanding (MOUs) with eight provinces facilitated progress on the clinical front, as they enable the supply of medicines and other health products to our clinics. The ninth MOU, for the Northern Cape, is in the pipeline.

CET COLLEGE PROGRAMME

The CET College Programme, which was introduced at the peak of the COVID-19 pandemic to create a safe studying environment for college students and staff, slumped in 2024 due mainly to the turnover of on-the-ground support staff from HIGHER HEALTH. Across the range of knowledge transfer, self-assessment and clinical service activities performance was well below target. The programme is being reviewed with the aim of rebuilding it in 2025.





SECTION 4

MANAGEMENT REPORT

HIGHER HEALTH experienced rapid growth in employee numbers from July 2024. By the end of March 2025, the total complement had increased to 223, which was 31% larger than the previous year. This expansion was part of a sustained growth period: over the past six years, our staff establishment has nearly trebled.

Most of the 2024/25 growth was due to the recruitment of 75 social workers to fulfil the roles of campus care workers and campus care coordinators in the TVET and CET college sub-sectors. A new organogram was introduced during the year and it provides for these employees to be managed by eight provincial offices.

In the 2025/26 financial year, a further 75 social workers will be engaged to provide support to the university sub-sector and once again the organisation will be called upon to adjust to the challenges of rapid growth.

As indicated below, individuals from previously disadvantaged groups are strongly represented in our staff complement at all occupational levels and we are proud to have retained our Level 2 broad-based black empowerment (B-BBEE) status. Our employees include two people with disabilities.

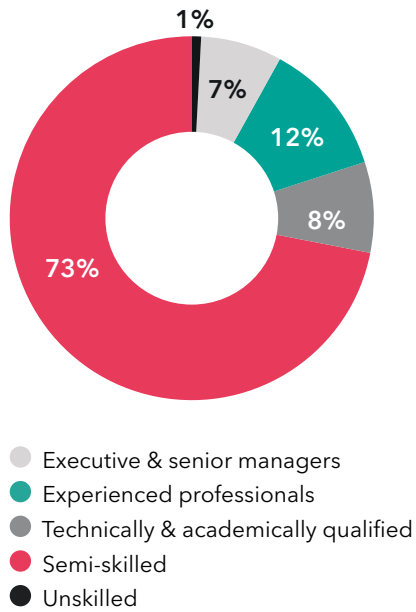
Stabilisation of our management team is critical to maintaining our organisational culture and knowledge base in the face of rapid growth. HIGHER HEALTH faces competition for skilled managers from both the public and non-profit sectors and has come to realise we must offer comparable remuneration and benefits. In the past year, we reviewed and adjusted the benefits we offer our permanent employees.

Throughout 2024/25, HIGHER HEALTH continued to strengthen its HR practices in accordance with our internal policies and statutory requirements.

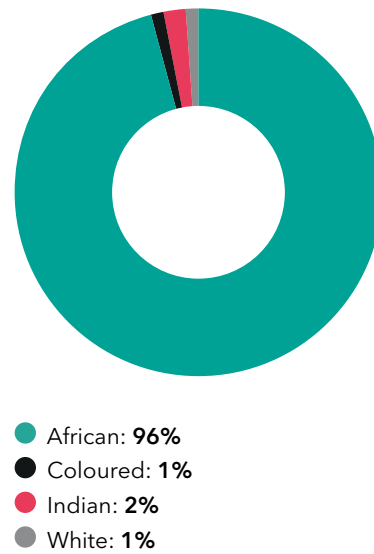
- In January, we introduced a performance management system featuring individual performance indicators aligned with the organisation's strategic plan.
- Our head office is now fully compliant with occupational health and safety requirements, and our objective is to ensure that all provincial offices soon achieve compliance.
- An internal audit indicated there were shortcomings in our management of employee leave and we switched digital leave management platforms to make this self-service facility more accessible to users and improve the accuracy of data capture.
- Our disciplinary procedures are being followed and, where necessary, employees are held to account for behaviour that undermines the performance of HIGHER HEALTH and its reputation.

The increase in employee numbers adds to the volume of routine HR administration tasks and HIGHER HEALTH is mindful that we need to ensure our HR team also has capacity to attend to more strategic HR activities, such as timely post-filling and employee satisfaction and retention

Occupational profile of employees



Employment equity in 2024/25



The gender ratio in 2024/25



2 CORPORATE COMMUNICATION

Our Communication Department continued to strengthen HIGHER HEALTH's visibility and engagement with stakeholders and to position the organisation as a national thought leader in youth empowerment and student health.

MASS MEDIA VISIBILITY

In 2024/25 we maintained a consistent presence in leading mass media, through unpaid editorial coverage on leading news channels - such as Newzroom Afrika, the SABC and eNCA - and targeted use of paid content placement. Prof Ahluwalia remains an important voice on matters related to youth health and development.

Our paid mass media exposure included partnerships with News24, TimesLive and Inside Education and focused on topics such as GBV and soft skills development.

IN-HOUSE DIGITAL PRODUCTION

During the past year we enhanced our ability to produce our own podcasts and other digital media and distribute these more systematically. For example, HIGHER HEALTH produced:

- A podcast by Prof Ahluwalia, titled Higher Health through the Lens.
- Prof Ahluwalia's 2025 student induction video.
- Four mental health videos to mark Mental Health Month.

- Livestreams from soft skills programme “camps” and videos of student testimonials.
- Videos related to the launch of Skills 4 Living in the UK.
- Promotional content, such as QR code-linked content and digital posters.

FOCUS ON DISTRIBUTION

The Communications Department also applied itself to the less exciting but crucial task of building digital communication channels. We used a variety of tactics:

- Developing our in-house database of internal and external stakeholders, which now enables HIGHER HEALTH to disseminate news regularly via MailChimp to more than 3 000 stakeholders.
- Collaborating with key strategic partners – such as DHET, the National Student Funding Assistance Scheme (NSFAS), universities and colleges – for the display of key digital materials on their platforms. We did this, for example, to ensure maximum reach and visibility for our student induction video in 2025.
- Paid digital campaigns, which can achieve massive exposure on carefully selected social media platforms. For example, the CEO’s Freedom Day message recorded more than 900 000 views and the combined views for the four mental health videos exceeded 2 million.

EVENTS ON-LINE AND IN-PERSON

2024/25 saw fewer HIGHER HEALTH events than previous years but they remained an important means of creating visibility and strengthening engagement. Major events that we helped organise and/or host during the year were:

- The annual Mandela Sobukwe Leadership Camp for peer educators representing universities and TVET colleges across the country. The camp occurs in Cape Town each February and in 2025 it was hosted by then Deputy Minister of Education and Training Buti Manamela.
- Events falling under the GBV campaign. From February 2025 Deputy Minister of Education and Training Mimmy Gondwe played a leading role in these, in collaboration with the Commission for Gender Equality.

- A webinar In March 2025 on the implications of the withdrawal of PEPFAR funding by US President Donald Trump. This was co-hosted with the Department of Health and the SA National AIDS Council and drew a total of 641 participants, indicating clearly the high level of concern about this development.

HIGHER HEALTH managers also presented to the annual conference of Marketing, Advancement and Communication in Education (MACE), held in Cape Town in November 2024.

MARKETING THE CIVIC AND HEALTH PEER EDUCATION PROGRAMME

As the Civic and Health Peer Education Programme moved from the pilot phase to national roll-out, the Communications Department mounted a campaign to market the programme to students nationwide. We generated promotional materials at the camps organised in late 2024 to incorporate classroom-based learning into the programme. The marketing campaign included:

- Lifestreaming from camps to a range of institutions.
- Producing 40 short videos in which students described how the programme had changed them.
- Reaching out via MailChimp to 4 000 students.
- Providing incentives in the form of digital devices for students to complete the programme.

LAUNCHING SKILLS 4 LIVING

Although the UK version of the soft skills programme, Skills 4 Living, is managed separately by HIGHER HEALTH UK, the South African body lends a hand wherever this is helpful. Our Communication Department played a pivotal role in planning, coordinating and executing the Skills 4 Living launch in the city of Manchester. Our team worked closely with counterparts at the Greater Manchester Combined Authority and the Office of the Mayor to ensure all media and promotional activities were aligned.

The launch attracted top-tier media coverage on BBC Radio and in The Guardian. The new HIGHER HEALTH UK website continues to serve as an inviting source of information on the programme, and testimonial videos made for the launch deliver value as credible marketing tools.



SECTION 5

GOVERNANCE

BOARD OF DIRECTORS

HIGHER HEALTH is a non-profit company, registered in terms of the Companies Act, and oversight of the organisation is vested in a Board of Directors.

The Board was constituted to ensure stakeholders that had been key to HIGHER HEALTH's development (Universities South Africa [USAf], South African Public Colleges Organisation [SAPCO] and the DHET) continued to play a role in our life and that

we had the range of expertise necessary for effective governance.

Prof Wim de Villiers retired at the end of March 2025 as Vice-Chancellor of Stellenbosch University and simultaneously stepped down as Chairperson of HIGHER HEALTH. He has been succeeded as Chairperson by Prof Deresh Ramjugernath, Vice-Chancellor of Stellenbosch University.

List of Board members serving in 2024/25

Board member	Designation	Organisation/sector represented or area of expertise
Prof Willem de Villiers Chairperson	Vice-Chancellor, Stellenbosch University	Universities/USAf
Prof Marianna Dipiloane Phutsisi Deputy Chairperson (Resigned January 2025)	Chairperson, SAPCO Principal, Motheo TVET College	TVET colleges/SAPCO
Mr TJ Kekana Deputy Chairperson (Assumed position May 2025)	Chairperson, SAPCO Principal, Sekhukhune TVET College	TVET colleges/SAPCO
Dr Precious Pethiwe Matutu	Chief Executive Officer, USAf	Universities/USAf
Ms Sesi Patricia Mahlobogoane	Director of Social Inclusion and Equity, DHET	PSET sector
Mr Xolelo B Phike (Resigned December 2025)	Principal, Northlink TVET College	TVET colleges/SAPCO
Ms T Futshane (Joined June 2024)	Deputy Director-General: CET Branch, DHET	DHET/CET sector
Prof Thokozile Valerie Mayekiso	Vice-Chancellor, University of Mpumalanga	Universities/USAf
Prof Nancy Refilwe Phaswana-Mafuya	Professor of Epidemiology and Public Health, University of Johannesburg	Public health
Dr MG Mothapo (Joined May 2025)	Principal, Sedibeng TVET College	TVET colleges/SAPCO
Dr Doeke Tromp	Former CFO, Tshwane University of Technology	Financial management and strategic planning
Dr (Prof) Ramneek Ahluwalia Ex officio	CEO, HIGHER HEALTH	

BOARD COMMITTEES

The Board has an Audit, Risk, Finance and Investment Committee (ARFIC), a Human Resources Committee, and a Remuneration Committee.

- The HR Committee and the Remuneration Committee comprise the same members. Mr Joseph Moloto of the University of Limpopo is the independent chair and Mr Hoffman Geyer of the University of Johannesburg serves as an independent HR advisor, with Board members Dr Matutu and Prof Phutsisi completing the committee.
- Prof Mayekiso, Dr Tromp, Mr CM Maimela (representing SACPO) and Mr E Manyawi (representing the TVET sector) are members of ARFIC, which has an independent chair, Mr Theo Madurai, CFO of Tshwane University of Technology.
- Prof Ahluwalia participates in meetings of ARFIC and the HR Committee. He attends the Remuneration Committee only when invited.

BOARD BUSINESS

At all meetings in 2024/25 the Board received a detailed progress report on the organisation's work during the preceding quarter and discussed related issues. In addition, the Board:

- Held its annual strategy session on 27 June 2024, where the possible expansion into the UK was deliberated and approved.
- Identified HIGHER HEALTH's considerable reliance on state funding as a potential risk and explored avenues to diversify funding sources.
- Approved the new organogram for the organisation, which provides for the establishment of the HIGHER HEALTH Academy.
- Reviewed and approved the annual financial statements, revised budget and annual report for 2023/24.
- Approved the operational plan and proposed budget for 2025/26.
- Filled Board vacancies as they occurred.
- Received regular reports on the establishment and progress of HIGHER HEALTH UK.

Participation in Board meetings 2024/25

	27 Jun 2024	5 Aug 2024	22 Nov 2024	28 Feb 2025
Prof de Villiers (Chair)	✓	✓	✓	✓
Prof Phutsisi	✓	✓	✓	Resigned
Prof Mayekiso	✓	✓	✗	✗
Dr Tromp	✓	✓	✓	✓
Mr Phike	✗	✓	✓	Resigned
Ms Mahlobogoane	✓	✓	✓	✓
Ms Futshane	✗	✓	✗	✗
Dr Matutu	✓	✓	✓	✓
Prof Phaswana-Mafuya	✓	✗	✗	✓
Mr Kekana	-	-	-	✓
Dr Mothapo	-	-	-	✗
Prof Ahluwalia	✓	✓	✓	✓





SECTION 6

EXCERPTS FROM ANNUAL
FINANCIAL STATEMENTS

GENERAL INFORMATION

ON HIGHER EDUCATION AND TRAINING HEALTH, WELLNESS AND DEVELOPMENT CENTRE

Country of incorporation and domicile	South Africa
Nature of business and principal activities	Health and wellness services and capacity development in higher education institutions which include universities and TVET colleges.
Directors	Prof W de Villiers (Chairperson) (Term ended 01/04/2025) Prof D Ramjugernath (Chairperson) (Appointed 17/06/2025) Dr (Prof) R Ahluwalia Dr D Tromp Prof N Mafuya Prof T Mayekiso Ms S Mahlobogoane (Appointed 24/06/2024) Dr P Matutu (Appointed 24/06/2024) Ms T Futshane (Appointed 24/06/2024) Prof D Phutsisi (Appointed 24/06/24. Term ended 14/01/25) Mr X Phike (Appointed 24/06/2024. Term ended 3/12/2024) Mr M Mothapo (Appointed 02/05/2025) Mr T Kekana (Appointed 02/05/2025)
Registered office	262 Rose Avenue Wild Olive Building, Riverfalls Office Park Centurion 0132
Business address	262 Rose Avenue Wild Olive Building, Riverfalls Office Park Centurion 0132
Postal address	262 Rose Avenue Wild Olive Building, Riverfalls Office Park Centurion 0132
Bankers	ABSA Bank Limited and Investec Bank Limited
Auditor	PricewaterhouseCoopers Inc
Company registration number	2017/419731/08
Tax reference number	9864/591/16/0
Level of assurance	These annual consolidated and separate financial statements have been audited in compliance with the applicable requirements of the Companies Act of South Africa.
Preparer	The annual consolidated and separate financial statements were independently compiled by Afrizen Accounting and Consulting (Pty) Ltd under the supervision of the Director of Finance, Yusuf Khan.

DIRECTORS' RESPONSIBILITIES AND APPROVAL

The directors are required by the Companies Act 71 2008 of South Africa to maintain adequate accounting records and are responsible for the content and integrity of the annual financial statements and related financial information included in this report. It is their responsibility to ensure that the annual financial statements satisfy financial reporting standards with regard to form and content and present fairly the statement of financial position, results of operations and business of the company, and explain the transactions and financial position of the business of the company at the end of the financial year, in conformity and compliance with the IFRS® Accounting Standards.

The annual financial statements are based upon appropriate accounting policies consistently applied throughout the company and supported by reasonable and prudent judgements and estimates.

The directors acknowledge that they are ultimately responsible for the system of internal financial control established by the company and place considerable importance on maintaining a strong control environment. To enable the directors to meet these responsibilities, the Board of Directors sets standards for internal control aimed at reducing the risk of error or loss in a cost-effective manner. The standards include the proper delegation of responsibilities within a clearly defined framework, effective accounting procedures and adequate segregation of duties to ensure an acceptable level of risk. These controls are monitored throughout the company and all employees are required to maintain the highest ethical standards in ensuring the company's business is conducted in a manner that, in all reasonable circumstances, is above reproach.

The focus of risk management in the company is on identifying, assessing, managing and monitoring all known forms of risk across the company. While operating risk cannot be fully eliminated, the company endeavours to minimise it by ensuring that appropriate infrastructure, controls, systems and ethical behaviour are applied and managed within predetermined procedures and constraints.

The directors of the company are responsible for establishing and controlling the process for electronically distributing annual reports and other financial information to stakeholders and the Companies and Intellectual Property Commission (CIPC).

The directors are of the opinion, based on the information and explanations given by management, that the system of internal control provides reasonable assurance that the financial records may be relied on for the preparation of the annual financial statements. However, any system of internal financial control can provide only reasonable, and not absolute, assurance against material misstatement or loss. The going-concern basis has been adopted in preparing the financial statements. Based on forecasts and available cash resources, the directors have no reason to believe that the company will not be a going concern in the foreseeable future. The financial statements support the viability of the company.

The external auditor is responsible for independently auditing and reporting on the company's financial statements. The financial statements have been examined by the company's external auditor and their report is presented on pages 8 to 10 [of the original annual financial statements].

The annual consolidated and separate financial statements and supplementary information set out on pages 11 - 36 and 37 respectively [of the original financial statements], including the Directors' Report and Audit, Risk, Finance and Investment Committee report set out on pages 4 to 5 and 6 to 7 respectively [of the original financial statements], which were prepared on the going concern basis, were authorised for issue and approved by the Board of Directors and were signed on their behalf by:



Prof D Ramjugernath
Chairperson



Dr (Prof) R Ahluwalia (CEO)
Chief Executive Officer

DIRECTORS' REPORT

The directors have pleasure in submitting their report on the annual consolidated and separate financial statements of the Higher Education and Training Health, Wellness and Development Centre NPC ("the company") for the year ended 31 March 2025.

1. Incorporation

The company was incorporated on 18 September 2017 and obtained its certificate to commence business on the same day.

2. Nature of business

Higher Education and Training Health, Wellness and Development Centre NPC was incorporated in South Africa with interests in health and wellness services and capacity development in higher education institutions which include universities and TVET colleges.

During the financial year, on 9 September 2024, HIGHER HEALTH (South Africa) registered a company called HIGHER HEALTH UK Ltd, in the United Kingdom. As at the reporting date, HIGHER HEALTH (South Africa), had not subscribed to any share capital in HIGHER HEALTH UK Ltd, and no financial transactions or investments have taken place.

These separate and consolidated annual financial statements have been prepared on the going concern basis in accordance with IFRS Accounting Standards as issued by the International Accounting Standards Board (IASB), and the requirements of the Companies Act of South Africa.

The operating results and statement of financial position of the company are fully set out in the attached financial statements and do not in our opinion require any further comment.

3. Directorate

The directors in office at the date of this report are as follows:

Director	Designation	Nationality
Prof W De Villiers (Chairperson)	Non-executive	South African (Term ended 01/04/ 2025)
Prof D Ramjugernath (Chairperson)	Non-executive	South African (Appointed 17/06/2025)
Dr D Tromp	Non-executive	South African
Dr (Prof) R Ahluwalia (CEO)	Executive	South African
Prof N Phaswana-Mafuya	Non-executive	South African
Prof T Mayekiso	Non-executive	South African
Ms Sesi Mahlobogoane	Non-executive	South African (Appointed 24/06/2024)
Dr P Matutu	Non-executive	South African (Appointed 24/06/2024)
Themisa Futshane	Non-executive	South African (Appointed 24/06/ 2024)

Director	Designation	Nationality
Prof D Phutsisi	Non-executive	South African (Term ended 14/01/ 2025)
Mr XB Phike	Non-executive	South African (Term ended 03/12/ 2024)
Mr MG Mothapho	Non-executive	South African (Appointed 02/05/2025)
Mr TJ Kekana	Non-executive	South African (Appointed 02/05/2025)

4. Directors' interests in contracts

During the financial year, no contracts were entered into which directors or officers of the company had an interest in and which significantly affected the business of the company.

5. Property, plant and equipment

There was no change in the nature of the property, plant and equipment of the company or in the policy regarding their use.

6. Events after reporting date

All events subsequent to the date of the annual financial statements and for which the applicable financial reporting framework requires adjustment or disclosure have been adjusted or disclosed.

7. Going concern

The annual consolidated and separate financial statements have been prepared on the basis of accounting policies applicable to a going concern. This basis presumes that funds will be available to finance future operations and that the realisation of assets and settlement of liabilities, contingent obligations and commitments will occur in the ordinary course of business.

The directors believe that the company has adequate financial resources to continue in operation for the foreseeable future and accordingly the annual consolidated and separate financial statements have been prepared on a going concern basis.

The ability of the company to continue as a going concern is dependent on a number of factors. The most significant of these is that the directors continue to procure funding for the ongoing operations of the company where the need arises. Management is comfortable that HIGHER HEALTH will continue to operate for the foreseeable future.

8. Independent Auditor

PricewaterhouseCoopers Inc continued in office as auditor for the year under review.

9. Secretary

No secretary has been formally appointed during the current financial year.

AUDIT, RISK, FINANCE AND INVESTMENT COMMITTEE REPORT

We are pleased to present our report for the financial year ended 31 March 2025.

The Audit, Risk, Finance and Investment Committee is an independent statutory committee appointed by the directors of the company. The committee has conducted its affairs in compliance with its terms of reference.

1. Members of the Audit, Risk, Finance and Investment Committee

The Audit, Risk, Finance and Investment Committee is independent and consists of five independent members. It meets at least twice per year as per its terms of reference.

The Chairman of the Board, Chief Executive Officer, Director of Finance, external auditor and other assurance providers (legal, compliance, risk, health and safety) attend meetings by invitation only.

The members of the Audit, Risk, Finance and Investment Committee are:

Name	Appointed	Institution
Mr T Madurai (Chairperson)	21/11/2017	Tshwane University of Technology, Chief Financial Officer
Prof T Mayekiso	21/11/2017	University of Mpumalanga, Vice-Chancellor
Dr D Tromp	21/11/2017	Independent financial consultant
Mr E Manyawi	15/03/2019	Sefako Makgatho Health Sciences University, Chief Financial Officer
Mr C Maimela	14/11/2023	Nkangala TVET College, Principal (Resigned: 03/12/2024)

2. Meetings held by the Audit Committee

The Audit, Risk, Finance and Investment Committee performs the duties laid upon it by section 94(7) of the Companies Act of South Africa by holding meetings with the key role players on a regular basis and by unrestricted access granted to the external auditors.

3. External auditor

The Audit, Risk, Finance and Investment Committee has satisfied itself that the external auditor was independent of the company, as set out in section 94(8) of Companies Act 71 2008 of South Africa, which includes consideration of previous appointments of the auditor, the extent of other work undertaken by the auditor for the company, and compliance with criteria relating to independence or conflicts of interest as prescribed by the Independent Regulatory Board for Auditors. Requisite assurance was sought and provided by the auditor that internal governance processes within the audit firm support and demonstrate its claim to independence.

The committee ensured that the appointment of the auditor complied with the Companies Act 71 of 2008 South Africa, and any other legislation relating to the appointment of auditors.

The Audit, Risk, Finance and Investment Committee, in consultation with executive management, agreed to the engagement letter, terms, audit plan and budgeted audit fees for the 2025 year.

4. Annual consolidated and separate financial statements

The Audit, Risk, Finance and Investment Committee has reviewed the accounting policies and the financial statements of the company and is satisfied that they are appropriate and comply with IFRS® Accounting Standards as issued by the International Accounting Standards Board (IASB®).

5. Effectiveness and internal controls

The system of internal control applied by the entity over financial reporting and risk management is effective, efficient and transparent. From the audit report on the financial statements and the management report of the external auditor, it was noted that no matters were reported that indicate any material deficiencies in the system of internal control or any deviations therefrom. Accordingly, the committee can report that the system of internal control over financial reporting for the period under review was efficient and effective.

On behalf of the Audit Risk Finance and Investment Committee:



T Madurai

Chairperson of the Audit, Risk, Finance and
Investment Committee

Date: Tuesday, 23 September 2025



Independent auditor's report

To the directors of Higher Education Training Health Wellness Development Centre NPC

Our opinion

In our opinion, the consolidated and separate financial statements present fairly, in all material respects, the consolidated and separate financial position of Higher Education Training Health Wellness Development Centre NPC (the Company) and its subsidiary (together the Group) as at 31 March 2025, and its consolidated and separate financial performance and consolidated and separate cash flows for the year then ended in accordance with IFRS Accounting Standards and the requirements of the Companies Act of South Africa.

What we have audited

Higher Education Training Health Wellness Development Centre NPC's consolidated and separate financial statements set out on pages 11 to 37 [of the original document] comprise:

- the consolidated and separate statement of financial position as at 31 March 2025;
- the consolidated and separate statement of comprehensive income for the year then ended;
- the consolidated and separate statement of changes in equity for the year then ended;
- the consolidated and separate statement of cash flows for the year then ended; and
- the notes to the financial statements, including material accounting policy information.

Basis for opinion

We conducted our audit in accordance with International Standards on Auditing (ISAs). Our responsibilities under those standards are further described in the *Auditor's responsibilities for the audit of the consolidated and separate financial statements* section of our report.

We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our opinion.

Independence

We are independent of the Group in accordance with the Independent Regulatory Board for Auditors' *Code of Professional Conduct for Registered Auditors* (IRBA Code) and other independence requirements applicable to performing audits of financial statements in South Africa. We have fulfilled our other ethical responsibilities applicable to performing audits in South Africa. The IRBA Code is consistent with the corresponding sections of the International Ethics Standards Board for Accountants' *International Code of Ethics for Professional Accountants (including International Independence Standards)*.

Other information

The directors are responsible for the other information. The other information comprises the information included in the document titled "Higher Education Training Health Wellness Development Centre NPC Operating as Higher Health formerly HEAIDS Consolidated and Separated Annual Financial Statements for the year ended 31 March 2025", which include(s) the Directors' Report and the Audit, Risk, Finance and Investment Committee Report as required by the Companies Act of South Africa. The other information does not include the consolidated or the separate financial statements and our auditor's report thereon.

Our opinion on the consolidated and separate financial statements does not cover the other information and we do not express an audit opinion or any form of assurance conclusion thereon.

In connection with our audit of the consolidated and separate financial statements, our responsibility is to read the other information identified above and, in doing so, consider whether the other information is materially inconsistent with the consolidated and separate financial statements or our knowledge obtained in the audit, or otherwise appears to be materially misstated.

If, based on the work we have performed, we conclude that there is a material misstatement of this other information, we are required to report that fact. We have nothing to report in this regard.

Responsibilities of the directors for the consolidated and separate financial statements

The directors are responsible for the preparation and fair presentation of the consolidated and separate financial statements in accordance with IFRS Accounting Standards and the requirements of the Companies Act of South Africa, and for such internal control as the directors determine is necessary to enable the preparation of consolidated and separate financial statements that are free from material misstatement, whether due to fraud or error.

In preparing the consolidated and separate financial statements, the directors are responsible for assessing the Group's and the Company's ability to continue as a going concern, disclosing, as applicable, matters related to going concern and using the going concern basis of accounting unless the directors either intend to liquidate the Group and/or the Company or to cease operations, or have no realistic alternative but to do so.

Auditor's responsibilities for the audit of the consolidated and separate financial statements

Our objectives are to obtain reasonable assurance about whether the consolidated and separate financial statements as a whole are free from material misstatement, whether due to fraud or error, and to issue an auditor's report that includes our opinion. Reasonable assurance is a high level of assurance, but is not a guarantee that an audit conducted in accordance with ISAs will always detect a material misstatement when it exists. Misstatements can arise from fraud or error and are considered material if, individually or in the aggregate, they could reasonably be expected to influence the economic decisions of users taken on the basis of these consolidated and separate financial statements.

As part of an audit in accordance with ISAs, we exercise professional judgement and maintain professional scepticism throughout the audit. We also:

- Identify and assess the risks of material misstatement of the consolidated and separate financial statements, whether due to fraud or error, design and perform audit procedures responsive to those risks, and obtain audit evidence that is sufficient and appropriate to provide a basis for our opinion. The risk of not detecting a

material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control.

- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of the Group's and the Company's internal control.
- Evaluate the appropriateness of accounting policies used and the reasonableness of accounting estimates and related disclosures made by the directors.
- Conclude on the appropriateness of the directors' use of the going concern basis of accounting and, based on the audit evidence obtained, whether a material uncertainty exists related to events or conditions that may cast significant doubt on the Group's and the Company's ability to continue as a going concern. If we conclude that a material uncertainty exists, we are required to draw attention in our auditor's report to the related disclosures in the consolidated and separate financial statements or, if such disclosures are inadequate, to modify our opinion. Our conclusions are based on the audit evidence obtained up to the date of our auditor's report. However, future events or conditions may cause the Group and / or Company to cease to continue as a going concern.
- Evaluate the overall presentation, structure and content of the consolidated and separate financial statements, including the disclosures, and whether the consolidated and separate financial statements represent the underlying transactions and events in a manner that achieves fair presentation.
- Plan and perform the group audit to obtain sufficient appropriate audit evidence, regarding the financial information of the entities or business units within the Group, as a basis for forming an opinion on the consolidated financial statements. We are responsible for the direction, supervision and review of the audit work performed for purposes of the group audit. We remain solely responsible for our audit opinion.

We communicate with the directors regarding, among other matters, the planned scope and timing of the audit and significant audit findings, including any significant deficiencies in internal control that we identify during our audit.

PricewaterhouseCoopers Inc.

PricewaterhouseCoopers Inc.

Director: R Ramdhany

Registered Auditor

Johannesburg, South Africa

23 September 2025

STATEMENT OF FINANCIAL POSITION

as at 31 March 2025

	2025	2024
	R'000	R'000
ASSETS		
Non-current assets		
Property, plant and equipment	9 040 488	8 181 821
Right-of-use assets	5 766 323	6 157 946
	14 806 811	14 339 767
Current assets		
Inventories	-	488 082
Trade and other receivables	3 982 153	7 870 871
Investments	21 821 460	19 271 663
Cash and cash equivalents	24 818 687	12 172 414
	50 622 300	39 803 030
Total assets	65 429 111	54 142 797
EQUITY AND LIABILITIES		
Equity		
Retained income	35 822 901	29 599 789
Liabilities		
Non-current liabilities		
Lease liabilities	4 190 669	5 509 024
Current liabilities		
Trade and other payables	6 859 794	3 945 586
Lease liabilities	2 377 702	1 622 619
Deferred income	14 992 166	11 180 811
Provisions	1 185 879	2 260 279
Cash and cash equivalents	-	24 689
	25 415 541	19 033 984
Total liabilities	29 606 210	24 543 008
Total equity and liabilities	65 429 111	54 142 797

STATEMENT OF PROFIT OR LOSS AND OTHER COMPREHENSIVE INCOME

as at 31 March 2025

	2025	2024
	R'000	R'000
Revenue	117 548 142	109 005 130
Other operating income	2 279 839	2 998 086
Other operating gains/(losses)	226 581	(160 819)
Operating expenses	(116 611 649)	(109 624 967)
Operating surplus	3 442 913	2 217 430
Interest income	1 038 620	28 020
Finance cost paid	(808 218)	(703 268)
Fair value gains	2 549 797	1 531 508
Surplus for the year	6 223 112	3 073 690
Other comprehensive income	-	-
Total comprehensive income for the year	6 223 112	3 073 690

HIGHER HEALTH is an approved public benefit organisation in terms of section 30 of the Income Tax Act (No 58 of 1962) and the receipts and accruals of HIGHER HEALTH are exempt from income tax in terms of section 10(1) (cN) of the Act.

STATEMENT OF CHANGES IN EQUITY

as at 31 March 2025

	Retained income	Total equity
	R'000	R'000
Balance at 01 April 2023	26 526 099	26 526 099
Total comprehensive income for the year 2024	3 073 690	3 073 690
Balance at 01 April 2024	25 599 789	29 599 789
Total comprehensive income for the year 2025	6 223 112	6 223 112
Balance at 31 March 2025	35 822 901	35 822 901

STATEMENT OF CASH FLOWS

as at 31 March 2025

	2025	2024
	R'000	R'000
Cash flows utilised in operating activities		
Cash generated from/(utilised in) operations	15 585 862	(4 614 212)
Interest income	1 038 620	-
Finance cost paid	(808 218)	(703 268)
Cash from operating activities: credit card facility	-	24 689
Net cash utilised in operating activities	15 816 264	(5 292 791)
Cash flows utilised in investing activities		
Purchase of property, plant and equipment	(1 569 252)	(1 682 154)
Proceeds from sale of property, plant, equipment	279 228	-
Net cash utilised in investing activities	(1 290 024)	(1 682 154)
Cash flows utilised in financing activities		
Cash repayments on lease liabilities	(1 879 967)	(1 903 257)
Total cash movement for the year	12 646 273	(8 878 202)
Cash and cash equivalents at beginning of year	12 172 414	21 050 616
Cash and cash equivalents at end of year	24 818 687	12 172 414

NOTES







HIGHER HEALTH

Higher Education & Training:
Health, Wellness and Development Centre



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